BREAST PROSTHESIS LOCATION RESOURCE LIST

THIS LIST IS INTENDED AS A RESOURCE TO ASSIST YOU IN FINDING A BREAST SUBSTITUTE (PROSTHESIS), BRA, LINGERIE OR SWIMWEAR AFTER YOU HAVE HAD A MASTECTOMY OR LUMPECTOMY. THIS LIST IS NOT AN ENDORSEMENT OR RECOMMENDATION OF ANY SPECIFIC ITEM OR STORE. IT IS NOT INTENDED TO BE A COMPLETE AND ACCURATE LISTING SINCE STORES AND PRODUCTS DO CHANGE. THIS LIST IS FOR GENERAL INFORMATION ONLY. CALL FOR STORE HOURS AND DAYS.

INFORMATION ON OBTAINING A PROSTHESIS AND/OR SPECIAL BRA(S):

1) IF YOU HAVE HAD SURGERY RECENTLY, CHECK WITH YOUR SURGEON TO MAKE SURE YOU ARE HEALED ENOUGH TO BE FITTED FOR A PROSTHESIS AND BRA. HEALING TIME IS REQUIRED SINCE YOU HAVE PAIN, TENDERNESS, AND SWELLING POST SURGERY. WHEN YOUR DOCTOR SAYS YOU ARE READY, AND ONLY THEN, SHOULD YOU BE FITTED WITH A BREAST SUBSTITUTE SUITED FOR YOU. THIS IS GENERALLY FOUR TO EIGHT WEEKS AFTER SURGERY.

2) BEFORE YOU TRY ON FORMS, CALL AND MAKE AN APPOINTMENT WITH A SUPERVISORY FITTER WHO HAS BEEN TRAINED TO FIT WOMEN WHO HAVE HAD BREAST SURGERY. TRY ON SEVERAL, FROM DIFFERENT MANUFACTURERS. THEY ALL FIT A LITTLE DIFFERENTLY, SO YOU WILL NEED TO EXPERIMENT TO GET THE ONE WITH THE BEST FIT AND FEEL FOR YOU. IT IS BEST TO TRY ON SEVERAL FORMS BEFORE YOU BUY ONE, SINCE MOST FORMS ONCE PURCHASED ARE NOT RETURNABLE.

3) SOME SALESPEOPLE ARE INCLINED TO MAKE SWEEPING AND UNFOUNDED CLAIMS OR TO USE PRESSURE. TAKE TIME TO EXPLORE AND DON'T BE UNDULY SWAYED BY THIS.

4) SOME WOMEN REQUIRE A BREAST FORM WITH UNDERARM EXTENSION. MOST FORMS ARE WEIGHTED IN VARIOUS WAYS TO BALANCE THE REMAINING BREAST. WHILE SOME SMALL BREASTED WOMEN CAN WEAR UNWEIGHTED FORMS, A LARGER BREASTED WOMAN NEEDS THE WEIGHT TO FEEL BALANCED AND TO HELP PREVENT BACK AND SHOULDER PROBLEMS. YOUR BRA SIZE WILL NOT BE THE SAME AS WHEN YOU WERE WEARING A REGULAR BRA DUE TO THE AMOUNT OF BREAST TISSUE REMOVED.

5) WEAR A FORM FITTING TOP WHEN TRYING ON THE PROSTHESIS SO YOU GET AN IDEA OF HOW NATURAL IT LOOKS.

6) WEAR THE FORM FOR ABOUT HALF AN HOUR WHILE IN THE STORE.

7) CHECK THE STORE'S RETURN POLICY AND READ THE CARE INSTRUCTIONS BEFORE PURCHASING.

8) THE PURCHASE OF THE PROSTHESIS IS COVERED UNDER MEDICARE. ONCE THE DEDUCTIBLE IS MET, MEDICARE WILL PAY 80% ON PROSTHESIS UP TO A MAXIMUM. IN THE STATE OF CALIFORNIA, ASSEMBLY BILL #3548, SECTION 1367.6, IT REQUIRES

THAT EVERY HEALTH CARE SERVICE PLAN CONTRACT IN CALIFORNIA, WHICH PROVIDES FOR A MASTECTOMY (PERFORMED AFTER JULY 1, 1980) SHALL PROVIDE COVERAGE FOR PROSTHETIC DEVICES AND RECONSTRUCTIVE SURGERY.

9) SOME PRIVATE MEDICAL INSURANCE POLICIES PAY FOR SPECIAL BRAS SO BE SURE TO LOOK INTO THIS. CALL YOUR MEMBERSHIP SERVICE DEPT. (LISTED ON THE BACK OF YOUR INSURANCE CARD) TO DETERMINE IF YOU HAVE BEEN ASSIGNED A SPECIFIC STORE TO GO TO OR IF YOU CAN GO ANYWHERE. THEY CAN ALSO TELL YOU THE NUMBER OF BRAS AND PROSTHESES THAT ARE COVERED.

10) THE COST OF FORMS AND BRAS WITH POCKETS ARE TAX DEDUCTIBLE ALONG WITH DRUGS AND MEDICAL EXPENSES. IF A CORSETIERE HAS TO PUT IN A POCKET OR ALTER A BRA, HER CHARGES ARE DEDUCTIBLE.

11) BE SURE THE BILLS AND CHECKS FOR PURCHASING SUCH ITEMS ARE MARKED "SURGICAL". TO INSURE BEING ABLE TO TAKE THE TAX DEDUCTIONS AND FOR INSURANCE COVERAGE, ASK YOUR SURGEON TO WRITE A PRESCRIPTION FOR YOUR PROSTHESIS AS FOLLOWS: SURGICAL PROSTHESIS AND BRAS FOR MASTECTOMY PATIENT. SALES TAX CANNOT BE CHARGE ON SURGICAL ITEMS.

12) CHECK WITH THE STORE TO SEE IF THEY WILL BILL YOUR INSURANCE OR IF YOU MUST PAY UP FRONT AND THEN IS REIMBURSED LATER.

13) THE AMERICAN CANCER SOCIETY HAS A PROSTHESIS BANK FOR WOMEN UNDER A FINANCIAL HARDSHIP. PLEASE CALL THEM FOR FURTHER INFO AT (909) 581-0687.

14) THE CANCER CARE CENTER AND THE AMERICAN CANCER SOCIETY CARRIES LIGHTWEIGHT FOAM PROSTHESES THAT CAN BE USED TEMPORARILY AFTER SURGERY UNTIL YOU HAVE HEALED. PLEASE CALL AND INQUIRE AT (909) 865-9691.

15) THE AMERICAN CANCER SOCIETY ALSO HAS A SUPPORT PROGRAM CALLED "REACH TO RECOVERY". THIS PROGRAM MATCHES YOU TO A BREAST CANCER SURVIVOR WHO HAS HAD SIMILAR SURGERY, AGE AND BACKGROUND TO GIVE YOU SUPPORT AND ANSWER QUESTIONS. PLEASE CALL THE AMERICAN CANCER SOCIETY FOR INFO. THIS IS A FREE PROGRAM AND DOES NOT REQUIRE A DOCTOR'S PRESCRIPTION.

Breast Prosthesis Location List

(Riverside June 2007)

Monrovia
Creative Women
1530 S. Myrtle Avenue
Monrovia, Ca 91016
(626) 358-6216

Will bill Medicare

Prosthesis- Classique (Partials)
Camp/Trulife, Amoena, Airway,
Bosom Buddy, Silima,
American Breast Care

Bras-Many brands
Swimwear- Best in local area

Bi-lingual fitters always available
On site sewing- will sew in pockets
and pads

Pomona
Pomona Surgical Supply
733 E. Holt Avenue
Pomona, CA 91767
(909) 623-4378

Will bill Medicare

Prosthesis- Airway , Jobst
Naturalwear, Freeman, Nearly Me,
Trulife, Jodee, Amoena
Active life

Bras- Airway, Amoena, Camp,
Jodee

No bathing suits
*Call for appt. with certified fitter.

Ontario
Hangar Orthopedic Group
3602 Inland Empire Blvd.
Ontario, CA 91764
(909) 484-5700

Prosthesis Amoeno, Camp
Bras- Amoena, Camp

Will take Medical
Will bill Medicare

No bathing suits
No sewing
*Call for appt. with certified fitter

Upland
The Bra Lady
958 N. Central Avenue
Upland, CA 91786
(909) 981-1741

Prosthesis- Nearly Me
Bras- Custom fit and add pockets

Regular bathing suits –will add Pockets

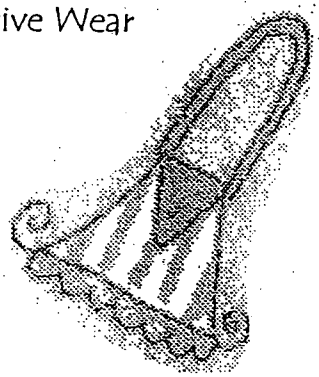
Does not BILL INSURANCE

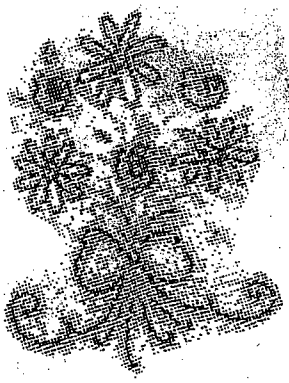
Large selection of lingerie- will add Pockets to nightgowns
* Call for appt. with certified fitter

Montclair
Nordstrom (Montclair Plaza)
Lingerie Dept.
5015 Montclair Plaza Lane
Montclair, CA 91763
(909) 625-0821

Prosthesis- Amoena
Bras- Amoena
If you buy a regular bra they will sew in a pocket for FREE

Can buy swimwear in Active Wear Dept.





Rancho Cucamonga
Fresh Peaches
12033 Jack Benny Dr. Suite 101
Rancho Cucamonga, CA 91739
(909) 980-0172
Hours Tuesday – Saturday
10:00-7:00

Make custom bathing suits.
Will also add pocket or pads.

Pasadena
Specialty Women's Health Shoppe
555 S. Lake AVENUE
Pasadena, CA 91101
(626) 792-8664

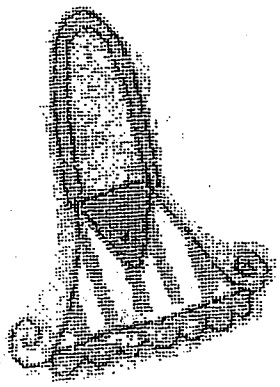
Prosthesis- Tru-life, Jodee
Amoena, Classique
Bras-Large variety

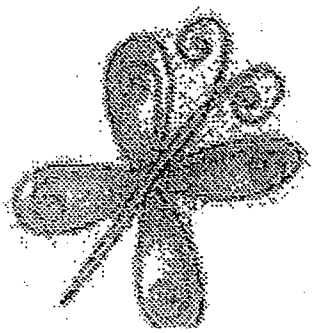
Will bill Medicare

Bathing Suits Limited
No Sewing
*Call for appt.
*Will also make home visits

Riverside
Nordstrom
Lingerie Dept.
The Galleria Mall
Riverside, CA 92506
(951) 351-3170

Prosthesis-Amoena
Bras- Amoena
Swimwear- Carries Swimwear in active Dept. Will sew pockets in for free on items bought there.





San Bernardino
BEEMAN'S BOUTIQUE
355 East 21st Street
San Bernardino, CA 92404
(909) 882-0193

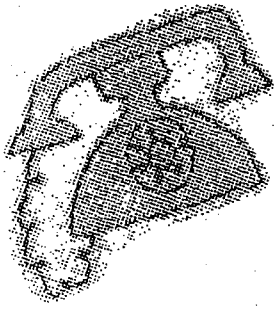
Will bill insurance

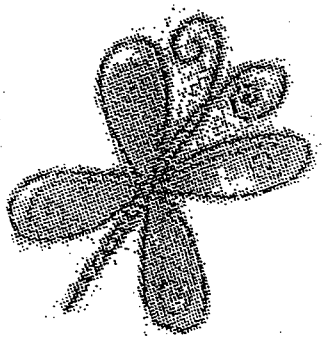
PROSTHESIS- Amoena, Bali Camp, American Breast Care

Bras- Variety

Bathing Suits- Carry a large Variety of suits all year and all sizes.

* Call for Appt with certified fitter.





GUIDELINES
FOR LOW-CHOLESTEROL, LOW-TRIGLYCERIDE DIETS
FOODS TO AVOID

MEATS, FISH	Marbled beef, pork, bacon, sausage, and other pork products; fatty fowl (duck, goose); skin and fat of turkey and chicken; processed meats; luncheon meats (salami, bologna); frankfurters and fast-food hamburgers (they're loaded with fat); organ meats (kidneys, liver); canned fish packed in oil.
EGGS*	Limit egg yolks to two per week.
FRUITS	Coconuts (rich in saturated fats).
VEGETABLES	Avoid avocados. Starchy vegetables (potatoes, corn, lima beans, dried peas, beans) may be used *only* if substitutes for a serving of bread or cereal. (Baked potato skin, however, is desirable for its fiber content.)
BEANS	Commercial baked beans with sugar and/or pork added.
NUTS	Avoid nuts. Limit peanuts and walnuts to one tablespoonful per day.
BREADS, GRAINS	Any baked goods with shortening and/or sugar. Commercial mixes with dried eggs and whole milk. Avoid sweet rolls, doughnuts, breakfast pastries (Danish), and sweetened packaged cereals (the added sugar converts readily to triglycerides).
MILK PRODUCTS	Whole milk and whole-milk packaged goods; cream; ice cream; whole-milk puddings, yogurt, or cheeses; nondairy cream substitutes.
FATS, OILS	Butter, lard, animal fats, bacon drippings, gravies, cream sauces, as well as palm and coconut oils. All these are high in saturated fats. Examine labels on "cholesterol-free" products for "hydrogenated fats." (These are oils that have been hardened into solids and in the process have become saturated.)
DESSERTS, SNACKS	Fried snack foods like potato chips; chocolate; candies in general; jams, jellies, syrups; whole-milk puddings; ice cream and milk sherbets; hydrogenated peanut butter.
BEVERAGES	Sugared fruit juices and soft drinks; cocoa made with whole milk and/or sugar. When using alcohol (1 oz liquor, 5 oz beer, or 2½ oz dry table wine per serving), one serving must be substituted for one bread or cereal serving (limit, two servings of alcohol per day).

SPECIAL NOTES

1. Remember that even nonlimited foods should be used in moderation.
2. While on a cholesterol-lowering diet, be sure to avoid animal fats and marbled meats.
3. While on a triglyceride-lowering diet, be sure to avoid sweets and to control the amount of carbohydrates you eat (starchy foods such as flour, bread, potatoes).
4. Buy a good low-fat cookbook, such as the one published by the American Heart Association.
5. Consult your physician if you have any questions.

SEE REVERSE SIDE FOR FOODS TO USE

GUIDELINES
FOR LOW-CHOLESTEROL, LOW-TRIGLYCERIDE DIETS
FOODS TO USE

MEATS, FISH	Choose lean meats (chicken, turkey, veal, and nonfatty cuts of beef with excess fat trimmed; one serving = 3 oz of cooked meat). Also, fresh or frozen fish, canned fish packed in water, and shellfish (lobster, crabs, shrimp, oysters). Limit use to no more than one serving of one of these per week. Shellfish are high in cholesterol but low in saturated fat and should be used sparingly. Meats and fish should be broiled (pan or oven) or baked on a rack.
EGGS	Egg substitutes and egg whites (use freely). Egg yolks (limit two per week).
FRUITS	Eat three servings of fresh fruit per day (1 serving = ½ cup). Be sure to have at least one citrus fruit daily. Frozen or canned fruit with no sugar or syrup added may be used.
VEGETABLES	Most vegetables are not limited (see reverse side). One dark-green (string beans, escarole) or one deep-yellow (squash) vegetable is recommended daily. Cauliflower, broccoli, and celery, as well as potato skins, are recommended for their fiber content. (Fiber is associated with cholesterol reduction.) It is preferable to steam vegetables, but they may be boiled, strained, or braised with polyunsaturated vegetable oil (see below).
BEANS	Dried peas or beans (1 serving = ½ cup) may be used as a bread substitute.
NUTS	Almonds, walnuts, and peanuts may be used sparingly (1 serving = 1 tablespoonful). Use pumpkin, sesame, or sunflower seeds.
BREADS, GRAINS	One roll or one slice of whole-grain or enriched bread may be used, or three soda crackers or four pieces of melba toast as a substitute. Spaghetti, rice, or noodles (½ cup) or ½ large ear of corn may be used as a bread substitute. In preparing these foods, do not use butter or shortening; use soft margarine. Also use egg and sugar substitutes. Choose high-fiber grains, such as oats and whole wheat.
CEREALS	Use ½ cup of hot cereal or ¾ cup of cold cereal per day. Add a sugar substitute if desired, with 99% fat-free or skim milk.
MILK PRODUCTS	Always use 99% fat-free or skim milk, dairy products such as low-fat cheeses (farmer's, uncreamed diet cottage), low-fat yogurt, and powdered skim milk.
FATS, OILS	Use soft (not stick) margarine; vegetable oils that are high in polyunsaturated fats (such as safflower, sunflower, soybean, corn, and cottonseed). Always refrigerate meat drippings to harden the fat and remove it before preparing gravies.
DESSERTS, SNACKS	Limit to two servings per day; substitute each serving for a bread/cereal serving: ice milk, water sherbet (¼ cup); unflavored gelatin or gelatin flavored with sugar substitute (½ cup); pudding prepared with skim milk (½ cup); egg white soufflés; unbuttered popcorn (1½ cups). Substitute carob for chocolate.
BEVERAGES	Fresh fruit juices (limit 4 oz per day); black coffee, plain or herbal teas; soft drinks with sugar substitutes; club soda, preferably salt-free; cocoa made with skim milk or nonfat dried milk and water (sugar substitute added if desired); clear broth. Alcohol: limit two servings per day (see reverse side).
MISCELLANEOUS	You may use the following freely: vinegar, spices, herbs, nonfat bouillon, mustard, Worcestershire sauce, soy sauce, flavoring essence.

1-2-3 Sensible Steps to Help You Manage Your Weight

Just as important as healthy eating is the need for increasing physical activity.

Step 3: Make physical activity as important as counting calories

Here are some examples of physical activities and the number of calories burned by each. Please note that the number of calories burned in an hour will vary depending on the activity, the intensity, and your weight and gender.

Walk to control your weight
Every 200 steps burns 10 calories. Burn 100 calories when you take an extra 2,000 steps (which is approximately one mile).

Include activity in your social plans with family and friends
Burn 290-450 calories with an hour of physical activity, depending on the activity.

Think of chores as part of your weight-control plan
Burn 180-330 calories when you do an hour of vigorous household chores, depending on the chore.

Calorie-Counter Activity Guide

Activity	Total Calories Used Per Hour*
Cooking	185-200
Walking Slowly (2½ mph)	210-230
Cleaning	235-355
Brisk Walking (4 mph)	250-345
Golf	300-350
Jogging (6 mph)	315-480
Cycling (9 mph)	315-480
Tennis	315-480
Skating	320-400
Gardening (heavy)	450-525
Basketball	480-625
Aerobic Dancing	480-625
Swimming	480-625
Cross Country Skiing	480-625

*... 150 pound person

Eat out and enjoy!

More and more people desire delicious, healthy meals when eating out, whether they are on a weight-management plan or not. Many restaurants now offer these healthy options on their menus, so you can enjoy eating out without jeopardizing your weight-management plan. If the meals are oversized, just cut back to a reasonable portion size.

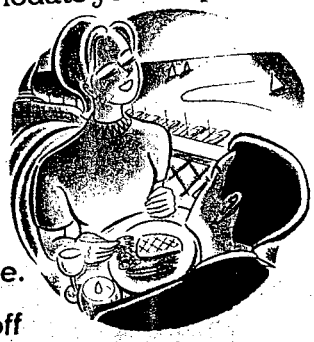
Be sure not to arrive too hungry to avoid overindulging. Eat a piece of fruit or have a large glass of water before you go out.

Cut out or cut down on extra bread or appetizers if you are having a main course.

Try to order first, so you are not tempted by what others order.

Avoid menu items that are very high in calories. For instance, foods that are fried or breaded, or contain cream, butter, or cheese.

Order foods that are broiled, baked, boiled, grilled, poached, steamed, roasted, or stir fried, without sauces. If they are not on the menu, many restaurants can accommodate your request if you ask.



Ask for dressings, sauces and spreads to be served "on the side" and limit your use.

Trim excess fat off meat; remove skin from chicken.

Remember: alcohol provides calories, but little or no nutrients.

As always, eat sensible portions. Restaurants will typically serve more food than you need; ask the waiter to wrap a portion to take home. At buffets, use a small plate and don't go back for second

Part of the SENSIBLE START TO WEIGHT CONTROL℠ sponsored by SPLENDA® No Calorie Sweetener

McNeil

© 2004 McNeil Nutritionals, LLC

1-2-3 Sensible Steps to Help You Manage Your Weight

Healthy eating is at the heart of effective weight control. This involves finding simple, healthy ways to reduce the number of calories you consume each day. You might be surprised at how small changes can make a big difference.

Step 1: Reduce calories with simple substitutions

Remember: added sugars add calories without vitamins and minerals. Here are a few ideas on how to cut back.

Save an average of 110-150 calories by drinking water or a no-calorie or low-calorie beverage instead of sugar-sweetened sodas.

Save an average of 190 calories by snacking on a small piece of fruit instead of a candy bar.

Save an average of 14 calories per teaspoon by switching to a non-caloric sweetener, such as SPLENDA® No Calorie Sweetener, to add to beverages and for baking.

Save an average of 140-280 calories when you eat smaller, sensible portions of your favorite foods.

Step 2: Size up sensible portions

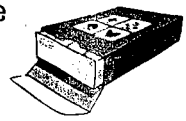
Consuming appropriate portion sizes from a variety of foods is the cornerstone of a sensible weight management plan. However, knowing the right portion size takes practice. It's easier at home, where measuring tools, a scale, and the same dinnerware are readily available. You will see and remember what the right portion looks like on your plate. Controlling your portion size can be more challenging when you are away from home. Use this guide to help you stay on your weight-management plan wherever you are.

What Counts as a Serving?

Bread, Cereal, Rice, and Pasta		
1 slice of bread	1 ounce of ready-to-eat cereal	1/2 cup of cooked cereal, rice, or pasta
Vegetable		
1 cup of raw leafy vegetables	1/2 cup of other vegetables, cooked or chopped raw	3/4 cup of vegetable juice
Fruit		
1 medium apple, banana, orange	1/2 cup of chopped, cooked, or canned fruit	3/4 cup of fruit juice
Milk, Yogurt, and Cheese (choose non-fat or low-fat)		
1 cup of milk or yogurt	1½ ounces of natural cheese	2 ounces of processed cheese
Meat, Poultry, Fish, Dry Beans, Eggs, and Nuts		
2-3 ounces of cooked lean meat, poultry, or fish	1/2 cup of cooked dry beans or 1 egg counts as 1 ounce of lean meat. 2 tablespoons of peanut butter or 1/3 cup of nuts count as 1 ounce of meat	

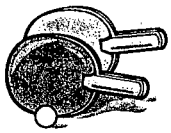
What sensible servings look like

Pair of dice = 1 ounce cheese



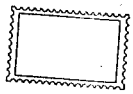
Deck of cards = 2 - 3 ounces meat

Checkbook = 3 ounces fish



Ping pong ball = 2 tablespoons peanut butter

Postage stamp = 1 teaspoon butter or margarine



Sample Daily Food Group Servings at 3 Calorie Levels

	1,600	2,200	2,800
Grain Group Servings	6	9	11
Vegetable Group Servings	3	4	5
Fruit Group Servings	2	3	4
Milk Group Servings	2-3	2-3	2-3
Meat Group (ounces)	5	6	7
Total Fat (grams)	53	73	93

Daily calorie needs are individualized. Speak to your healthcare provider about your appropriate calorie intake level for weight management.

Splenda® No Calorie Sweetener

See reverse side for Step 3

Part of the SENSIBLE START TO WEIGHT CONTROL℠ Program, sponsored by SPLENDA® No Calorie S...

Eating right for a healthier heart

Eating fewer high-fat, high-cholesterol foods and watching calories are important steps to a healthier diet and a healthier heart.

Guidelines for a healthy diet

With your doctor, determine the number of calories you need each day to achieve or maintain a desirable weight.

Eat less high fat food (especially those high in saturated fat).

Replace part of the saturated fat in your diet with unsaturated fat.

Less than 30% of your total daily calories should come from fat (less than 10% of your calories should come from saturated fat, no more than 10% of your calories should come from polyunsaturated fat; 10% to 15% of your calories should come from monounsaturated fat).

Control your daily cholesterol intake—eat less than 300 mg a day.

Choose foods high in complex carbohydrates (starch and fiber).

If you drink alcohol, do so in moderation.

Saturated fat is fat from animal and whole milk dairy products, as well as from some oils (for example, coconut, butter, cream, cheese, and palm oil).

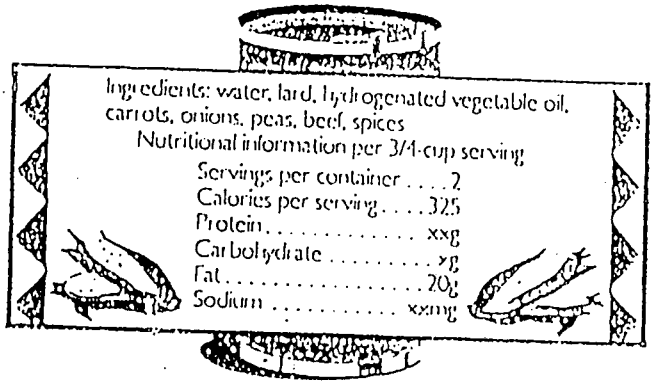
Unsaturated fat is a type of fat that is usually a liquid and is not as bad for you as saturated fat.

Neither saturated fat and polyunsaturated fat are two kinds of unsaturated fat.

Monounsaturated fat is a slightly unsaturated fat found in foods from plants, including olive and canola (rapeseed) oils.

Polyunsaturated fat is a highly unsaturated fat found in foods from plants, including safflower, sunflower, corn, and soybean oils.

Read the labels to find the fat



Ask yourself these questions to help choose products that are low in fat, especially products that are low in saturated fat.

1. Is fat a major ingredient?
On the label, all ingredients are listed by weight. The ingredient in the greatest amount is listed first. The ingredient in the least amount is listed last. To avoid too much fat and cholesterol, go easy on products that list any ingredient high in saturated fat or cholesterol first.

On this product, lard and hydrogenated fat are listed early in the ingredients; therefore, this is a high fat product.

2. Is there more than one type of fat in the product?
When you see several high-fat ingredients on a label (such as lard, hydrogenated vegetable oil, and beef), the product probably has too much fat.

3. Is the serving size appropriate?
This label says there are two 3/4-cup servings in this can—a serving that may be less than what you would usually eat. When you are figuring out how much fat there is in something, make sure to use a serving size that is close to what you would really eat.

1 gram fat = 9 calories

If a serving has 5 grams of fat, then:

5 × 9 = 45 calories from fat

If the total calories of the serving are 100, then:

45/100 = 45% of calories from fat

Eating out

Be just as careful when you dine out as you are when you make your own meals!

Salad	Use light corn oil and vinegar dressings
Bread	Use margarine instead of butter if desired
Chicken	Eat broiled or grilled chicken; remove its skin before eating
Steak	Ask the chef to trim off any excess fat before cooking
Hamburger	Avoid cheese or mayonnaise toppings—add tomato or lettuce instead
Fish	Select broiled or poached fillets
Pasta	Use marinara, clam, or tomato sauce without meat or sausage (avoid cream sauce)
Pizza	Choose vegetable toppings instead of cheese or meat toppings
Baked potato	Avoid toppings such as butter or sour cream
Vegetables	Eat plenty of these, but without any heavy sauces
Dessert	Sherbet and fresh fruit are excellent choices
Coffee, tea	Use skim milk or nondairy, nonfat creamer

Choose your low-cholesterol, heart-healthy diet

To get the nutrients you need, you have to eat a variety of foods. One way to do this is to choose foods from the different food groups. After determining your dietary goals with your doctor, adjust the number and size of portions to reach and stay at your desired weight.

	Choose	Go Easy	Avoid
Meat, poultry, fish, and shellfish (up to 6 ounces/day)	Lean cuts of meat with fat trimmed, chicken and turkey without skin, fish, shellfish		"Prime"-grade fatty cuts of meat, goose, duck, liver, kidneys; sausage, bacon, regular luncheon meats, hot dogs
Dairy products (2 servings/day; 3 for pregnant or breastfeeding women)	Skim milk, 1% milk, low-fat buttermilk, low-fat evaporated or nonfat milk, low-fat yogurt, cottage cheese, cheeses labeled "no more than 2 to 6 grams of fat per ounce"	2% milk, yogurt, part-skim ricotta, part-skim or imitation hard cheeses (like part-skim mozzarella), "lite" cream cheese, "lite" sour cream	Whole milk, cream, half and half, imitation milk products, whipped cream, custard-style yogurt, whole-milk ricotta, hard cheeses (like Swiss, American, cheddar, muenster), cream cheese, sour cream
Eggs	Egg whites, cholesterol-free egg substitutes	Egg yolks (no more than 3/week)	
Fats and oils (up to 6 to 8 teaspoons/day)	Corn, olive, peanut, canola (rapeseed), safflower, sesame, and soybean oils, tub (not stick) margarine	Nuts, seeds, avocados, olives	Butter, lard, bacon fat, coconut and palm kernel oils
Breads, cereals, pasta, rice, dried peas, and beans (6 to 11 servings/day)	Most breads, bagels, English muffins, rice cakes, low-fat crackers (like matzo, bread sticks, rye krisps, saltines); hot and cold cereals; spaghetti, macaroni, noodles, and any grain rice; dried peas and beans	Store-bought pancakes, waffles, biscuits, muffins, and cornbread	Croissants, sweet rolls, danish, doughnuts, and crackers made with saturated oils; granola-type cereals made with saturated oil, egg noodles, pasta, and rice prepared with cream, butter, or cheese sauces
Fruits and vegetables (2 to 4 servings of fruit/day; 3 to 5 servings of vegetables/day)	Fresh, frozen, canned, or dried fruits		Vegetables prepared in butter, cream, or sauce
Snacks (avoid too many sweets)	Sherbet, sorbet, Italian ice, frozen yogurt, popsicles, angel food cake, fig bars, gingersnaps, low-fat jelly beans and hard candy, plain popcorn, pretzels, fruit juices, tea, coffee	Ice milk, fruit crisps and cobblers, homemade cakes, cookies, and pies prepared with unsaturated oils	Ice cream, frozen tofu, candy, chocolate, potato chips, buttered popcorn, milkshakes, frappes, floats, eggnog, store-bought pies, most store-bought frosted and pound cakes

PARKE-DAVIS

BOOTS PHARMACEUTICALS

A medical service of PARKE-DAVIS Division of Warner-Lambert Company, Morris Plains, New Jersey 07950

© 1991 Warner-Lambert Company

Choose your low-cholesterol, heart-healthy diet

To get the nutrients you need, you have to eat a variety of foods. One way to do this is to choose foods from the different food groups. After determining your dietary goals with your doctor, adjust the number and size of portions to reach and stay at your desired weight.

	Choose	Go Easy	Avoid
Meat, poultry, fish, and shellfish (up to 6 ounces/day)	Lean cuts of meat with fat trimmed, chicken and turkey without skin, fish, shellfish		"Prime"-grade fatty cuts of meat, goose, duck, liver, kidneys; sausage, bacon, regular luncheon meats, hot dogs
Dairy products (2 servings/day; 3 for pregnant or breastfeeding women)	Skim milk, 1% milk, low-fat buttermilk, low-fat evaporated or nonfat milk, low-fat yogurt, cottage cheese, cheeses labeled "no more than 2 to 6 grams of fat per ounce"	2% milk, yogurt, part-skim ricotta, part-skim or imitation hard cheeses (like part-skim mozzarella), "lite" cream cheese, "lite" sour cream	Whole milk, cream, half and half, imitation milk products, whipped cream, custard-style yogurt, whole-milk ricotta, hard cheeses (like Swiss, American, cheddar, muenster), cream cheese, sour cream
Eggs	Egg whites, cholesterol-free egg substitutes	Egg yolks (no more than 3/week)	
Fats and oils (up to 6 to 8 teaspoons/day)	Corn, olive, peanut, canola (rapeseed), safflower, sesame, and soybean oils, tub (not stick) margarine	Nuts, seeds, avocados, olives	Butter, lard, bacon fat, coconut and palm kernel oils
Breads, cereals, pasta, rice, dried peas, and beans (6 to 11 servings/day)	Most breads, bagels, English muffins, rice cakes, low-fat crackers (like matzo, bread sticks, rye krisps, saltines); hot and cold cereals; spaghetti, macaroni, noodles, and any grain rice; dried peas and beans	Store-bought pancakes, waffles, biscuits, muffins, and cornbread	Croissants, sweet rolls, danish, doughnuts, and crackers made with saturated oils; granola-type cereals made with saturated oil, egg noodles, pasta, and rice prepared with cream, butter, or cheese sauces
Fruits and vegetables (2 to 4 servings of fruit/day; 3 to 5 servings of vegetables/day)	Fresh, frozen, canned, or dried fruits		Vegetables prepared in butter, cream, or sauce
Snacks (avoid too many sweets)	Sherbet, sorbet, Italian ice, frozen yogurt, popsicles, angel food cake, fig bars, gingersnaps, low-fat jelly beans and hard candy, plain popcorn, pretzels, fruit juices, tea, coffee	Ice milk, fruit crisps and cobblers, homemade cakes, cookies, and pies prepared with unsaturated oils	Ice cream, frozen tofu, candy, chocolate, potato chips, buttered popcorn, milkshakes, frappes, floats, eggnog, store-bought pies, most store-bought frosted and pound cakes

PARKE-DAVIS

BOOTS PHARMACEUTICALS

A medical service of PARKE-DAVIS Division of Warner-Lambert Company, Morris Plains, New Jersey 07950

© 1992 Warner-Lambert Company

Eating right for a healthier heart

Eating fewer high-fat, high-cholesterol foods and watching calories are important steps to a healthier diet and a healthier heart.

Guidelines for a healthy diet

With your doctor, determine the number of calories you need each day to achieve or maintain a desirable weight.

Eat less high fat food (especially those high in saturated fat).

Replace part of the saturated fat in your diet with unsaturated fat.

Less than 30% of your total daily calories should come from fat (less than 10% of your calories should come from saturated fat, no more than 10% of your calories should come from polyunsaturated fat; 10% to 15% of your calories should come from monounsaturated fat).

Control your daily cholesterol intake—eat less than 300 mg a day.

Choose foods high in complex carbohydrates (starch and fiber).

If you drink alcohol, do so in moderation.

Saturated fat is fat from animal and whole milk dairy products, as well as from some oils (for example, coconut, butter, cream, cheese, and palm oil).

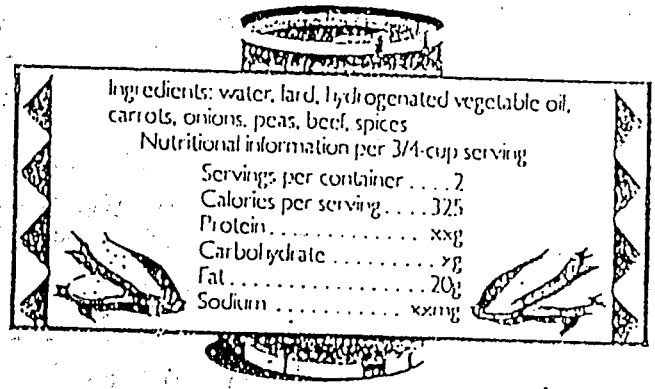
Unsaturated fat is a type of fat that is usually a liquid and is not as bad for you as saturated fat.

Monounsaturated fat and polyunsaturated fat are two kinds of unsaturated fat.

Monounsaturated fat is a slightly unsaturated fat found in foods from plants, including olive and canola (rapeseed) oils.

Polyunsaturated fat is a highly unsaturated fat found in foods from plants, including safflower, sunflower, corn, and soybean oils.

Read the labels to find the fat



Ask yourself these questions to help choose products that are low in fat, especially products that are low in saturated fat.

1. Is fat a major ingredient?
On the label, all ingredients are listed by weight. The ingredient in the greatest amount is listed first. The ingredient in the least amount is listed last. To avoid too much fat and cholesterol, go easy on products that list any ingredient high in saturated fat or cholesterol first.

On this product, lard and hydrogenated fat are listed early in the ingredients; therefore, this is a high fat product.

2. Is there more than one type of fat in the product?
When you see several high-fat ingredients on a label (such as lard, hydrogenated vegetable oil, and beef), the product probably has too much fat.

3. Is the serving size appropriate?
This label says there are two 3/4-cup servings in this can—a serving that may be less than what you would usually eat. When you are figuring out how much fat there is in something, make sure to use a serving size that is close to what you would really eat.

1 gram fat = 9 calories

If a serving has 5 grams of fat, then:

5 × 9 = 45 calories from fat

If the total calories of the serving are 100, then:

45/100 = 45% of calories from fat

Eating out

Be just as careful when you dine out as you are when you make your own meals!

Salad	Use light corn oil and vinegar dressings
Bread	Use margarine instead of butter if desired
Chicken	Eat broiled or grilled chicken; remove its skin before eating
Steak	Ask the chef to trim off any excess fat before cooking
Hamburger	Avoid cheese or mayonnaise toppings—add tomato or lettuce instead
Fish	Select broiled or poached fillets
Pasta	Use marinara, clam, or tomato sauce without meat or sausage (avoid cream sauce)
Pizza	Choose vegetable toppings instead of cheese or meat toppings
Baked potato	Avoid toppings such as butter or sour cream
Vegetables	Eat plenty of these, but without any heavy sauces
Dessert	Sherbet and fresh fruit are excellent choices
Coffee, tea	Use skim milk or nondairy, nonfat creamer

Lori L. Vanyo, M.D.
138 Harvard Avenue
Claremont, CA 91711
(909) 624-4503

Patient Name _____

MINOR PROCEDURE AFTERCARE INSTRUCTIONS

☐ If you have a dressing, leave it in place for 24 hours after the procedure. Keep the wound dry.

☐ You may shower after _____ days.

☐ Avoid aspirin or aspirin-containing products for _____ hours after the procedure (unless essential for other medical reasons). Tylenol may be taken for discomfort.

☐ Occasionally a wound may ooze. If this occurs, change the dressing as needed.

☐ If bleeding should occur, apply steady pressure using 1 or 2 fingertips and a small gauze pad for 20 minutes. Do not lift the gauze to check the site until the 20 minute period is over (or bleeding may restart). This usually stops any bleeding. If bleeding can not be controlled, **call us at (909) 624-4503.**

☐ There will normally be a small area of redness around the wound as it heals. However, please contact us if excessive redness, excessive tenderness, excessive discharge or fever develops, which may indicate an infection.

☐ Please make an appointment for follow-up / suture removal in ____ to____ days. Be sure to keep your appointment.

☐ You may use silicone gel sheets, such as Mepiform, to aid in the healing and fading of scars _____ weeks after your procedure.

It may take 4 – 6 weeks for the wound to heal entirely and 6 – 24 months for the scar to flatten and the redness to fade. (It takes longer on the trunk and legs). Wear sunscreen whenever the wound site is exposed.

Please call our office at (909) 624-4503 if you have any problems or questions.

WILSHIRE ONCOLOGY
MEDICAL GROUP, INC.

Hot Flash Management

By Traci Young, MSN, RN, ONP &
Linda Bosserman, MD, FACP

Hot flashes are experienced by 80% of all women when estrogen levels are low secondary to chemotherapy, surgical menopause, or the discontinuation of supplemental estrogen because of breast cancer. Hot flashes are described as a sudden sensation of heat or burning starting in the head and neck area, passing in waves over the entire body, accompanied by profuse sweating, sensation of pressure in the head, like a headache, and occasionally shaking (tremors) and palpitations. Although hot flashes may occur at any time of the day, some women may experience it more at night, causing them to be awakened from sleep. This may be due to the stress or excess stimuli we experience when having a dream. The exact scientific process leading to hot flashes is not known.

Hot flashes may occur any time but tend to begin during the peri-menopausal period as women approach menopause, and their menses to become irregular. We define menopause as the time after a women stop having periods, and can be associated with highly variable levels of estrogen. When periods stop after hysterectomy (surgical removal of the uterus only), the ovaries may continue to produce normal amounts of estrogen for long periods of time. Some women continue to have enough estrogen produced by their ovaries or adrenal glands throughout their lives and may experience mild, brief or no hot flashes. Other women are profoundly affected by hot flashes that can occur many times a day and wake patients several times at night. Regardless of the severity of hot flashes, they will usually become less intense and less frequent over 3-6 months for most patients. Some patients, unfortunately, can experience severe hot flashes for years.

In addition to hot flashes, other symptoms associated with low estrogen levels and menopause include: memory impairment, sleeplessness, mood swings, depression, vaginal dryness, lack of libido (interest in sex) and painful intercourse due to vaginal dryness. Like hot flashes, some women never experience any of these symptoms, while others may have a few of these symptoms and they resolve after several months. Some women may have symptoms to the point that their daily life is drastically impacted. The treatments for these symptoms need to be individualized. A variety of treatments have the potential to treat several symptoms, making them a good choice when more than one symptom occur.

Menopausal symptoms are all common and normal, especially for young women that become suddenly menopausal after necessary chemotherapy treatments. Women should be made to feel comfortable in discussing these issues with their doctors and nurse practitioners, especially after breast cancer where many factors need to be taken into account to find the best treatments. Women need to feel free to follow-up with their specialty team between visits if treatments are not working and adjustments are necessary. Several visits and a great deal of patience may be needed to find the best regimen for an individual patient. Having a treatment team committed to your symptom management, and who are experienced in this area of care, can markedly enhance your quality of life.

Several approaches are utilized in hot flash management, starting with practical tips, nutritional supplements, non-prescription remedies and prescription medications. For many

women, hormone replacement with Estrogen, Progesterone or Testosterone products relieves the many symptoms of menopause, including hot flashes. For women who have had breast cancer, non-hormonal treatments are the first line of care. Women who have had breast cancer, however, do have a right to make an informed consent choose to use hormonal therapies. This is done when persistent, intolerable hot flashes or other menopausal symptoms are not relieved by non-hormonal therapies and quality of life is impaired. The choice to use hormone therapies after a diagnosis of breast cancer is made after a full discussion of the many controversies regarding possibly increasing the risk of new or recurrent breast cancer versus the quality of life benefits with hormone treatments. In cases where women choose to use hormone-based therapies to control intolerable menopausal side effects, the lowest doses and best routes are reviewed in detail with each patient individually.

Hot Flash Management Recommendations Based on Scientific Findings

Lifestyle changes

- Dress in layers
- Drink plenty of cool water

Avoid triggers:

- Internal-anxiety, stress
- External-alcohol, spicy foods, caffeine,
- Close physical contact
- Stress reduction through meditation,
- Guided imagery, affirmations

Non-Pharmacological Treatments

Vitamin C:

500 mg tablets, 1 tablet twice a day for a total of 1000mg. This is a good recommendation for health as well.

Vitamin E:

400iu tablets, 1 tablet twice a day for a total of 800iu, although it has been recommended for treating hot flashes since the 1940s, use of it was no more effective than placebo in decreasing hot flash frequency or severity. One study does indicate that there is statistically significant, but minor reduction in hot flash equal to one hot flash per person per day, one less than observed with a placebo. Some patients, though, get great relief with minimal cost and side effects so it is worth a try.

Peridan C:

2 tablets, three time a day for 2 weeks; 1 tablet three times a day for 1 month; 1 tablet every day. (Patients have reported lack of availability in last several months) This is basically high dose Vitamin C but, again, it works great for some patients.

Soy Protein:

25-50 gm per day (about four servings of soy foods), available in milk or powder form, or soy beans at Trader Joe's. Controversy about possible breast cancer stimulation has been raised but studies in Japanese women consistently show a low rate of breast cancer with high soy diets which stops when Japanese women come to America and decrease their soy intake. Soy protein is best for complete protein benefits, the use of pills with the most active phyto-estrogens is without data for benefit although some patients have reported hot flash control with the pills.

Black Cohosh (Remifemin):

20 mg one tablet, twice a day. Both clinical trials studying the use of black cohash showed a significant effect on menopausal symptoms compared to placebo, typically 60-70% improvement over 12 weeks, with no estrogenic effect on vaginal epithelium, endocrine hormone levels, or endometrial thickness.

***There are many other herbal remedies thought to be useful in the treatment of hot flashes such as evening primrose oil, dong quai and wild yam cream, however there are no clinical trials that have indicated any efficacy in the use of these remedies.*

Pharmacologic-Non Hormonal Therapies, all by prescription

Bellargal S:

1 tablet twice a day. If excessive sleepiness occurs:

- Start out taking ½ tablet at bedtime for one week, then
- If hot flashes persist, increase to 1 tablet once a day at bedtime.
- If hot flashes continue, take 1 tablet in the in the morning and 1 tablet at bedtime for one week.
- If still having hot flashes, take 2 tablets in the morning and 2 at bedtime for one week.
- Should the hot flashes remain uncontrollable, notify your healthcare provider. (**Should not be taken if you have peripheral vascular disease, coronary artery disease, blood infections, glaucoma, are pregnant or nursing, take with caution if you have asthma. Caution when taking in combination with narcotics, antidepressants, and phenothiazines, will cause excessive drowsiness**).

This medication can be taken along with several others if more control is needed.

Effexor XR:

75 mg, once a day (This is generally very well tolerated. Some women experience dry mouth, decreased appetite, or mild nausea. These symptoms generally resolve after the second weeek. Benefits are usually seen after the third to fourth week.) A study by Charles Loprinzi of the Mayo Clinic Cancer Center concluded that the 75 mg dose reduces hot flashes by ~60% from baseline in breast cancer patients.

- This is usually started as 37.5 mg orally each day for a week to minimize any side effects.
- This dose can be taken long term but 75 mg is the usual effective dose and is started after the first week.
- Doses can be increased to 150mg/day if needed.
- This is excellent for control of mood swings and depression which often accompanies menopause. This drug and others like it do not give you a drug, rather it prevents breakdown of your natural seratonin levels which are responsible for our moods. Higher levels of natural seratonin result in lifting of depression and control of mood swings, panic attacks and anxiety attacks. Popular media have misled people when they imply taking this type of medication is just a "happy pill". When seratonin levels are normal, people do not feel better with these medications and they don't take them!

Clonidine:

0.1 mg once a day or twice a day (available as pill or transdermal patch in three strengths: TTS-1, TTS-2 and TTS-3 to be used for a week at a time), in a study by Charles Loprinzi, 116 women using the transdermal patches experienced a 20% greater reduction in hot flash frequency than with placebo. Similarly, Kishan Pandya, M.D. at the University of Rochester Cancer Center Community Clinical Oncology Program Research Base reported that after 8 weeks of treatment with oral clonidine at 0.1 mg per day,

breast cancer patients experienced a 38% mean decrease in the frequency of hot flashes. Side effects include dry mouth, constipation and drowsiness, which generally resolve after 1-3 weeks.

Megace:

20-40 mg once a day, women taking this has reported a 75% reduction in hot flashes. Doses can be adjusted to 40mg twice a day or up to 80 mg twice a day. Results occur in 4-6 weeks. Doses can be decreased once hot flashes are controlled to the lowest effective levels needed for hot flash control. Many women refuse or stop taking this medication due to increase in their appetite resulting in weight gain that can be 10 pounds per year. If other methods are not helpful, using Megace should receive strong consideration as it is an effective anti-breast cancer medication and the doses can be reduced to minimize the appetite increase.

Paxil:

10-20mg once a day. In a clinical trial at the Georgetown University, Vered Stearns, M.D. reported in a pilot trial of 20 women that 20mg of Paxil led to a mean reduction in hot flash frequency of 67%. The mean reduction in severity of hot flash was 75%. A low occurrence of side effects was reported, somnolence was the most common complaint. Placebo controlled trials are in progress. This medication is in the same general category of SSRI's with Effexor, Prozac, and Zoloft. If side effects are found with one drug in this category, another in the category may be effective without that side effect and is worth considering. These medications all help with mood swings, panic disorders, anxiety attacks, and depression, all of which can accompany menopause.

Neurontin:

300 mg two or three times per day. The dose can be started as low as 100mg twice a day and you can increase every 3-5 days up to 300 mg twice per day, then 300 mg three times per day or 600 mg twice a day. The maximum dose is 3600 mg/day so it is extremely well tolerated. This medication is on the market for seizures and also helps with pain control. Many experts have reported significant hot flash relief for women. The main side effect is sleepiness, which wears off after the first two weeks. Slowly increasing the dose can help minimize this effect.

Dilantin:

300 mg orally at night: This seizure medication can be effective for some patients. It is relatively inexpensive but can have side effects long term and requires drug level monitoring by blood tests. Several drugs can raise or lower the blood levels and interactions need to be watched for.

References:

Berendsen, H.G. (2000). The role of serotonin in hot flushes. Maturitas, 36, 155-164.

Carpenter, J.S. (2000). Managing Hot Flashes After Breast Cancer. Oncology Nursing Forum, 27 (1), 23-24.

Eden, J.A. (2001). Herbal medicines for menopause: do they work and are they safe? Medical Journal of Australia, 174, 63-64.

Fighting scar wars? Let the silicone be with you

"This article was originally published in the *Wound Care* newsletter, November 97.
Reprinted with permission of American Health Consultants, PO Box 740056, Atlanta, GA 30374
Telephone (800) 688-2421"

(Mepiform) silicone gel sheet

By **Liza G. Ovington**, PhD
Program Director, Wound and Continence Management
Home Health Division, Southeast Florida
Columbia Healthcare Corp
Fort Lauderdale, FL

Gel sheets work, but no one knows why

Silicone gel sheeting is a material that has evolved not for the treatment of open draining wounds but for the sometimes extreme results of wound healing in certain patients: hypertrophic or keloid scars. Since the early 1980s, researchers and clinicians have been investigating the beneficial effect of silicone gel sheets on scars. These benefits are manifested as any or all of the following changes in the scar:

- a color change (toward that of surrounding skin);
- a texture change (softening of the scar, increased pliability);
- a change in size (scar tissue mass decreases).

The earliest studies were performed in burn patients but subsequent studies have documented similar beneficial effects on scar formation after surgery, trauma, or various chronic diseases. The therapy appears to work on patients with fresh scars as well as those with scars of long duration (up to 12 years).

In general, the patient applies the silicone gel sheet directly to the scar, without pressure, and leaves it in place for eight to 12 hours per day for several months. The silicone gel sheets look similar to clear gelatin. Most of them are naturally sticky and will stay in place on skin fairly well. However, different types of tape can be used to anchor them if desired. Because the gel sheet is placed onto intact, albeit scarred skin, there are no absorbent demands placed on the silicone. In fact, silicone gel sheets are nonabsorbent. When used for scar therapy, the gel sheets may be washed gently by hand with mild soap and water and reapplied multiple times. Over time, the silicone gel may break up, especially if placed in a high friction area or picked at.

Why gel sheeting works remains a mystery

The mechanisms by which the silicone gel sheeting achieves its positive effects on scarring are not yet well understood. Originally, the sheets were used beneath pressure garments in burn scar therapy, and it was thought that pressure alone accounted for the positive effects, with the silicone gel just serving to evenly distribute the pressure. This was subsequently shown not to be true when the silicone gel sheets alone without external pressure application achieved the same effects on scars.

Later it was postulated that the silicone gel sheets were releasing silicone into the scar tissue, and somehow that generated the effect. Research studies showed that this, too, was not the case. It has been well documented that there are no silicone monomers or extracts available from the gel sheeting form of the product (unlike liquid silicone in breast implants).

Then it was thought that the silicone sheet was simply providing occlusion of the scar tissue and that the subsequent hydration of the tissue was responsible for the change in the scar. Research comparing scar occlusion by a silicone gel sheet vs. occlusion by a hydrocolloid dressing or hydration via a moisturizer

documented that only the silicone gel sheet changed the nature of the scar.[1] However, another study compared occlusion of scars by silicone gel sheets vs. occlusion by hydrogel sheets and found similar beneficial effects from both materials.[2]

To date there is no clear evidence to explain the therapeutic effect of silicone gel on scar tissue. However, many studies have documented the end result. The products have been prospectively shown to minimize scar development as well as achieve changes in scars of long duration. (Examples of commercially available silicone gel sheeting products are shown in the table below.)

Silicone Gel Sheeting Manufacturers		
BRAND NAME	MANUFACTURER	INFORMATION
Silon-SES Silicone Elastomer Sheeting	Bio Med Services	(800) 257-4566
Novagel Silicone Gel Sheeting	Brennen Medical	(800) 328-9105
Cica-Care Silicone Gel Sheeting	Smith & Nephew	(800) 876-1261
Mepitel (silicone- coated netting)	SCA Molnlycke	(800) 992-9939

References:

1. Phillips TJ, et al. A randomized controlled trial of hydrocolloid dressing in the treatment of hypertrophic scars and keloids. *Dematol Surg* 1986; 22:775-778.
2. Ricketts CH, et al. Cytokine mRNA changes during the treatment of hypertrophic scars with silicone and nonsilicone dressings. *Dermatol Surg* 1996; 22:955-959.

Gout Food Lists

Recommended Foods

The chart shows foods that are not high in purines. You can eat any amount of the foods that are low in purine — choose healthy foods most often. For the foods that are moderate in purines, stick to the amounts shown in the chart.

Food Group	Foods low in purines	Foods moderate in purines
Beverages	Water, juice, carbonated beverages, tea, coffee, cocoa	
Breads and Cereals	Breads, pastas, rice, cakes, cornbread, popcorn	Oatmeal (do not eat more than 2/3 cup uncooked, daily). Wheat bran, wheat germ (do not eat more than ¼ cup dry, daily)
Condiments	Salt, herbs, olives, pickles, relishes, vinegar	
Dairy	All dairy foods (low-fat or nonfat types are best)	
Fats and Oils	All types except gravies and sauces made with meat	
Fruits	All	
Protein Foods	Eggs, nuts, peanut butter	Meat and poultry. Crab, lobster, oysters, and shrimp (limit to 1-2 servings* daily). Dried beans, peas, lentils (limit to1 cup cooked daily).
Soup	made without meat	Meat- or fish-based soups, broths, bouillons
Vegetables	All vegetables except those that are moderate in purines	Asparagus, cauliflower, spinach, mushrooms, green peas (do not eat more than ½ cup of these vegetables per day)
Other Foods	Sugar, sweets, gelatin	

* 1 serving = 2 to 3 ounces.

Copyright American Dietetic Association. This handout may be duplicated for client education.

Gout Sample 1 Day Menu

Breakfast	Cereal with low-fat or nonfat milk
	Whole wheat toast with butter or margarine and jam
	Fruit juice
	Coffee
Lunch	Peanut butter sandwich on whole wheat bread with banana or jam
	Fruit crisp
	Milk
Evening	Cheese lasagna
	Green lettuce with tomatoes
	French bread
	Fruited Jello
	Nonalcoholic beverage

* Snacks can be added to this menu. Choose healthy snack foods, and stay within the daily caloric range recommended by you dietitian.

Copyright American Dietetic Association. This handout may be duplicated for client education.

Sivaprasad Mullangi, M.D.
138 Harvard Avenue
Claremont. CA 91711
Phone: (909) 624-4503 Fax: (909) 624-6364

SURGICAL INSTRUCTIONS

*** IMPORTANT TO REVIEW ***

1. Surgery date _____

2. Please pre-admit at Medical Center 3 – 4 days prior to your surgical procedure: Monday if your surgery is on Thursday, Thursday if your surgery is on Monday, or Friday if your surgery is on Tuesday. Have your bloodwork done on the same day.

3. The hospital will call you the day before your surgery to notify you of your arrival time. If they have not called by 5:00 P.M., call them at

4. Do NOT take any blood-thinning medications such as aspirin or any products that contain aspirin for seven (7) days prior to your surgery date.

5. Nothing to eat or drink after midnight the night before your surgery.

6. If you have a bowel prep, have nothing but clear liquids the entire day prior to the surgery. Start the prep on _____ at _____.

7. Take all blood pressure medication with a tiny sip of water early in the morning; all other medications, please check with the prescribing doctor.

8. Please make plans for someone to drive you home after your surgical procedure.

9. If you have any questions, please call (909) 624-4503 and ask for Dr. Mullangi's nurse.

10. Please call (909) 624-4503 a day or two after your surgery to make your post-op appointment with Dr. Mullangi. This appointment should be 7-10 days after your surgery.

Thank you.

Chaparral Medical Group
INCORPORATED

Donna E. Dudziak, M.D., F.A.C.S.
Diplomate American Board of Surgery
General, Minimally Invasive & Bariatric Surgery

POSTOPERATIVE DISCHARGE INSTRUCTIONS

1. For 2 weeks, no lifting, carrying, pulling, or pushing anything heavier than 10 pounds. Do not do anything strenuous – ***if it hurts, don't do it***.

2. No driving while taking pain medicine. ***Do not take any Tylenol*** while taking the prescription pain medicine. You may take Ibuprofen as directed on the bottle.

3. You may take an over-the-counter stool softener (like Colace or Peri-Colace) while taking pain medicine to prevent constipation.

4. Do not get incision(s) wet for 48 hours. After 48 hours remove the outer dressing if there is one – leaving the little steri-strips intact. After 48 hours you may shower daily, washing wounds gently with mild soap and water. Pat dry and leave alone. For 1 week take only showers, do not bathe, swim, or submerge. The steri-strips will fall off in 3-5 days. You can remove them if they are still intact after 5 days.

5. Call the office and schedule an appointment to be seen on/in_____

6. Call the office if you have any questions or concerns, especially if your incision becomes hot, red, painful, or begins to drain.

SPECIAL INSTRUCTIONS:

Drains – record output each 24 hours
bring this information to your appointment

Open wounds – change as directed

138 Harvard Ave. • Claremont, CA 91711 • (909) 624-4503 • Fax (909) 624-6364

Donna Dudziak, MD, FACS
138 Harvard Ave.
Claremont, CA 91711
(909) 624-4503
Fax: (909) 626-8384

Surgical Instructions

Surgery Date: _____

1. Please pre-admit at _____ 3-4 days prior to your surgical procedure.

2. The hospital will contact you the day prior to your surgery, regarding your arrival time to the hospital.

3. Seven (7) days prior to your surgery, you MUST DISCONTINUE taking all blood thinning medication(s) such as, Aspirin or any product that contains Aspirin.

4. Nothing to eat or drink after midnight, the night before your surgery.

5. Blood pressure medication(s) should be taken as usual with just a sip of water; for all other routine medication(s) please check with me.

6. You MUST make arrangements for someone to drive you home from the hospital.

7. If you have any questions/concerns, please call my office and you will be assisted.

Thank you,

Donna Dudziak, MD, FACS

HIGH FIBER DIET FOR DIVERTICULAR DISEASE

Purpose: To provide a diet with 3 to 4 grams of appropriate fiber to promote regular elimination and increases fecal excretion.

Use: This diet can be used for the management of diverticulosis.

Dietary Guidelines

1. Omit nuts, fruits and vegetables with seeds because they may become lodged in pockets protruding from the intestinal wall
2. Coarse bran is the most effective fiber source in helping food move through the digestive system faster.
3. Increase consumption of whole grains, legumes, fruits and vegetables without seeds.
4. Drink at least 6 to 8 cups of fluid each day
5. Eat a balanced diet including foods from the basic four food groups

HIGH FIBER DIET FOR DIVERTICULOSIS

HIGH FIBER CHOICES	AVOID FOLLOWING FOODS
BREADS & GRAINS 6-11 servings each day	BREADS & GRAINS
Whole grain breads, muffins, and bagels without seeds, nuts or coconut	Any breads with seeds, nuts, or coconut.
Pita bread, tortillas	
Whole wheat crackers or crisp breads without seeds	
Whole grain or bran cereals without nuts or coconut	
Oatmeal, oat-bran, or grits	
Wheat germ	
Whole wheat pasta	
Brown rice	
FRUIT 2-4 servings each day	FRUIT
All fruits without seeds such as apples, apricots (fresh or dried), banana, avocado, dates, honeydew melon, kumquats, papaya, prunes, pumpkin, orange, peach, pear, grapefruit, nectarine	Fruits with seeds such as blackberries, blueberries, strawberries, raspberries, whole cranberries, cranberry relish, figs, pomegranates, grapes, raisins with seeds

	Jam and preserves with seeds
VEGETABLES 3-5 servings each day Vegetables without seeds such as asparagus, artichokes, broccoli, Brussels sprouts, cabbage, carrots, green beans, green pepper, onions, peas, potatoes with skin, snow peas, spinach, mature squash(except zucchini), sweet potatoes, parsnips	**VEGETABLES** Vegetable containing seeds such as tomatoes, cucumbers, and zucchini squash; corn, celery.
MILK & DAIRY 2-3 servings each day All kinds of milk Cheese, yogurt or ice cream that does not contain seeds or nuts	**MILK & DAIRY** Cheese, yogurt or ice cream containing seeds or nuts
MEAT & MEAT SUBSTITUTES 2-3 servings or total of 6 oz daily All meat, poultry, fish, and eggs Beans and peas such as garbanzo beans, kidney beans, lentils, lima beans, great northern, pinto, white navy, split peas, and pinto beans Soybeans Smooth and creamy peanut butter	**MEAT & MEAT SUBSTITUTES** Chunky style peanut butter Sausage with seeds Nuts and seeds, whole or coarsely chopped, such as almonds, peanuts, Brazil nuts, cashew: peanut butter, walnuts, sesame and sunflower seeds
FATS & SNACKS (use sparingly) Whole-wheat pretzels Cakes, breads, and cookies made with oatmeal, fruit without nuts, coconut or seeds	**FATS & SNACKS** Pickles, relishes Popcorn Jam and preserves with seeds

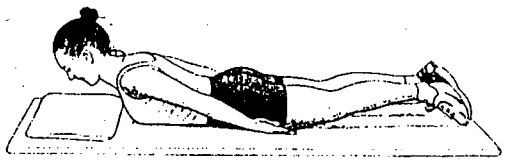
Exercise vs Arthritis

"Muscle is the most important protector of joints," says Tufts University exercise expert Miriam Nelson. "Cartilage only absorbs shock that gets *past* the muscle. Without strong muscle, cartilage wears out much sooner."

These two exercises, adapted from Nelson's "Strong Women and Men Beat Arthritis" (Penguin Putnam, 2002), can help strengthen the muscles in your lower back, buttocks, and hamstrings.

For the exercise that uses weights, exhale as you lift the weight and inhale as you lower it, even though that's the opposite of what most people would do intuitively.

Back Extension



1. Lie face down with your forehead resting on the floor, legs stretched back, toes pointing back. Stretch back your arms on either side of your body, palms facing up. Tuck your tailbone under you slightly.

2. As you inhale, raise your chest off the floor, keeping your head, neck, and back all in line (see illustration). Don't lift your legs off the floor. Hold the position for 3-5 seconds.

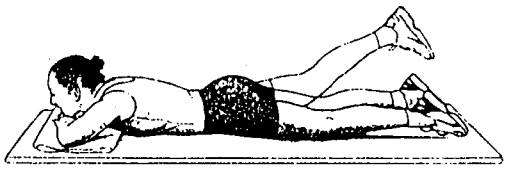
3. Exhale as you lower back to the ground. Keep your shoulders back and your chest open as you rise up and lower down.

Reps and sets: Do 5 repetitions. Rest for 15 seconds between each rep.

As the exercise becomes easier:

1. Hold the lifted position for 10 seconds.
2. Hold for 20 seconds.

Hip Extension



1. Lie on the floor on your stomach with your legs extended, toes pointing back. Place your arms in front of your head and rest your head on your hands with your forehead facing down. (You can use a firm bed for this exercise if you're unable to get down on the floor.)

2. Raise your right leg from your hip joint, keeping your leg straight and your trunk stable, toes pointing down toward the ground (see illustration). Initiate the movement from your hip and the back of your thigh. Pause for 1 second when you feel a squeeze in the buttock muscle. (If you feel any pain or a pinch in your lower back, you have lifted the leg too high. Lower it slightly until the pain subsides.)

3. Lower your right leg to the floor.

Reps and sets: Complete 12 repetitions on one side and then repeat on the other side—this is 1 set. Rest for about 30 seconds and then do a second set.

As the exercise becomes easier:

1. When you can do 12 repetitions in good form, add ankle weights, in 1-pound increments per workout, until the intensity reaches a four.*
2. As you become stronger and the intensity falls below a four, increase the weight by 1- to 2-pound increments each workout until you are at a four again.

* *Intensity of Four: Hard effort. By the time you have completed 6 or 7 repetitions, the exercise has become difficult. While you should be able to do the exercise 12 times in good form, you need to rest afterward.*

Adapted from "Strong Women and Men Beat Arthritis," © 2002.
Used by permission of G.P. Putnam's Sons, a division of Penguin Group (USA) Inc.

Upper-body strength test ①

Age	Above average	Average	Below average
Women			
20-29	>32	26-32	<26
30-39	>29	23-29	<23
40-49	>27	21-27	<21
50-59	>25	20-25	<20
60-69	>22	19-22	<19
70-79	>21	18-21	<18
80-89	>19	16-19	<16
Men			
20-29	>39	35-39	<35
30-39	>36	32-36	<32
40-49	>34	30-34	<30
50-59	>33	29-33	<29
60-69	>31	26-31	<26
70-79	>28	24-28	<24
80-89	>24	20-24	<20

① *Number of repetitions.*

weight in your stronger hand. (You can use either a dumbbell or a 1-gallon plastic jug filled with 5 pints of water or 5 pounds of sand.)

② Hold your arm straight down at your side, palm facing forward.

③ Keeping your upper arm stationary, do biceps curls: Bend your arm at the elbow and raise the weight to your shoulder, then lower it.

④ Count how many repetitions you can do in 30 seconds. Then check the chart above.

Lower-body test:

① Strap a 10-pound ankle weight onto the ankle of your stronger leg, or measure a total of 10 pounds of sand or small stones into two old socks and secure them to your stronger ankle.

② Sit in a chair on enough cushions to keep your feet off the ground, or sit on a table or countertop.

③ Keeping your back straight and your thigh stationary, raise your foot by straightening the knee, then bring the foot back down.

④ Do as many repetitions as you can in 30 seconds. Then consult the chart below.

Lower-body strength test ①

Age	Above average	Average	Below average
Women			
20-29	>29	26-29	<26
30-39	>28	25-28	<25
40-49	>27	24-27	<24
50-59	>27	23-27	<23
60-69	>25	22-25	<22
70-79	>25	22-25	<22
80-89	>22	19-22	<19
Men			
20-29	>33	29-33	<29
30-39	>32	28-32	<28
40-49	>31	27-31	<27
50-59	>30	27-30	<27
60-69	>29	25-29	<25
70-79	>28	23-28	<23
80-89	>26	21-26	<21

① *Number of repetitions.*

Flexibility test ①

Age	Above average	Average	Below average
Women			
20-29	>20½	17½-20½	<17½
30-39	>19½	16½-19½	<16½
40-49	>19	14½-19	<14½
50-59	>17½	14½-17½	<14½
60-69	>17	14-17	<14
70-79	>16½	13-16½	<13
80-89	>15	12½-15	<12½
Men			
20-29	>17	13½-17	<13½
30-39	>16½	13-16½	<13
40-49	>16	12½-16	<12½
50-59	>15½	12-15½	<12
60-69	>14	10-14	<10
70-79	>12	9½-12	<9½
80-89	>10	7½-10	<7½

① *Measurements in inches.*

Flexibility test

Tight muscles can hamper your everyday activities, limit your athletic performance, and make you more susceptible to soreness and injury. This test evaluates how flexible the muscles are in the back of your legs, your buttocks, and your lower back. If you're stiff there, you'll probably be stiff in other parts of your body, too.

① Tape a yardstick to the floor, then place a foot-long strip of tape perpendicular to the stick, to flag the 15-inch mark.

② Remove your shoes and sit on the floor with the yardstick between your extended legs, the low end toward you. Keep your heels about a foot apart, on the 15-inch mark. Place one hand over the other, with middle fingers aligned.

③ Lower your head and slide your fingers forward as far as you can without bending your knees, straining, or jerking. (See the illustration above.)

④ Record the farthest point where you can hold the tips of your middle fingers for two seconds. Do the test three times, then check your best score against the chart above.

Balance test

Just as strength, endurance, and flexibility tend to deteriorate as people age, so does the sense of balance. And poor balance coupled with increasing stiffness raises the risk of debilitating falls. It's preferable to have someone time you for this test, although you could time yourself. (Frail individuals should have someone stand nearby in case they start to fall.)

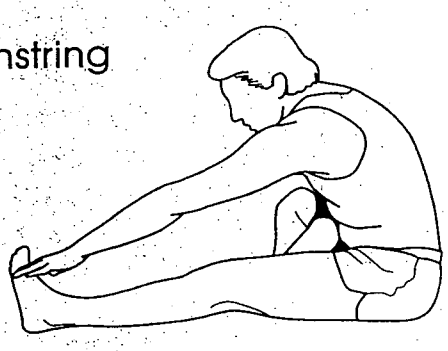
① If you're age 50 or older, you'll need a stopwatch or a stopwatch feature to measure your time to a tenth of a second. Younger people can measure their time in whole seconds if they want, then round off the numbers in the chart to the nearest second (since those numbers are high enough to round off without distorting the results).

② Raise the foot of your weaker leg and balance on the other foot, keeping your eyes open and your arms hanging limp at your sides.

③ Time how long you can keep your foot raised. Do the test three times, then check the best result against the chart on the facing page.

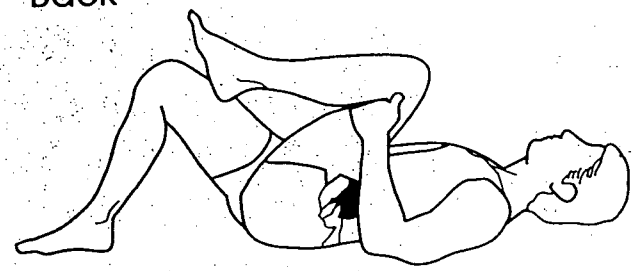
Flexibility Exercises

Hamstring



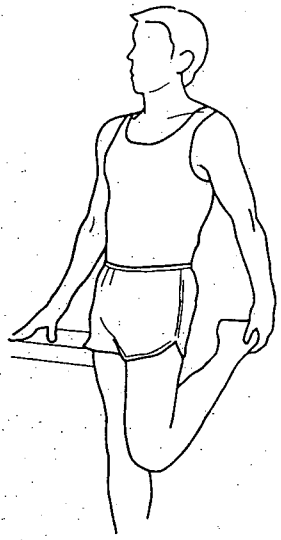
Reach down along leg until a comfortable stretch is felt in back of thigh. Be sure to keep knee straight. Hold 10 seconds.

Back



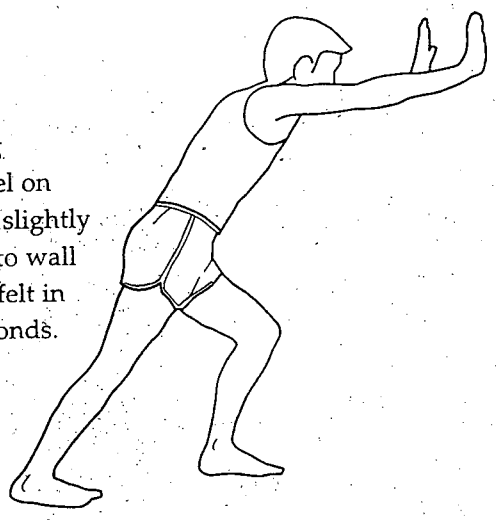
Hand behind (under) knee
Maintain pelvic tilt. Bend both knees and alternate bringing one knee to chest. Hold 5 seconds. Repeat 10 times each leg.

Quadriceps



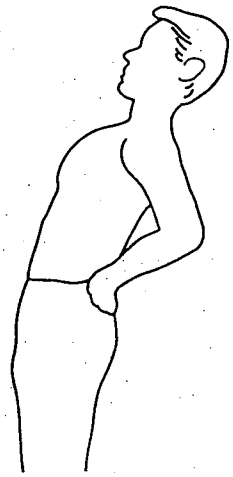
Pull heel toward buttock until a stretch is felt in front of thigh. Hold 10 seconds.

Calves



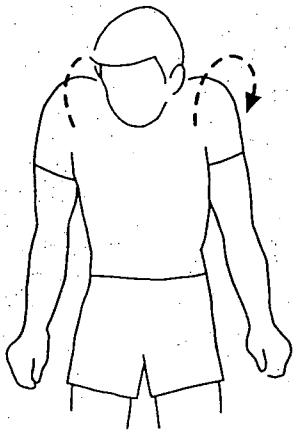
Keeping back leg straight, with heel on floor and turned slightly outward, lean into wall until a stretch is felt in calf. Hold 10 seconds.

Back



Stand upright and place you hands in the small of your back. Bend backwards at the waist, keeping your knees as straight as possible and return to the upright position. Repeat 10 times.

Shoulder Rolls



Pull your shoulders forward, upward and then backward, rolling them in a full circle. Repeat 10 times.

Exercise for the neck and shoulders

exercise for the neck and shoulders

Exercise is an important adjunct to your treatment. However, be sure to follow your doctor's instructions carefully. Do not overdo exercise, particularly when beginning. Start by trying the movements slowly and carefully. If the exercise causes some mild discomfort, which lasts a few minutes, do not be alarmed. This will disappear as your muscles become stronger. But, if pain is more than mild and continues for more than 15 to 20 minutes, discontinue exercising and speak to your doctor.

General Information

Always do the exercises in the order marked by your doctor. (Please note: Consult your doctor before doing these or any other exercises.)

Instructions: _____

_____

_____

_____

Prescriber: _____ Date: _____

1 HEAD TURN: Stand or sit erect. Keep back straight. Slowly turn head as far as possible to the right. Hold position to the count of 5. Return to normal position. Repeat this procedure to opposite side. Repeat 5 times.

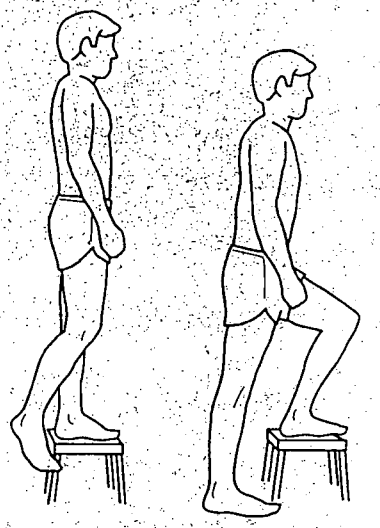
2 CHIN TUCK AND LIFT: Stand erect. Slowly lower chin trying to touch chest. Hold position to the count of 5. Slowly raise head back, looking up to the ceiling. Do not force or strain during this exercise. Hold position to the count of 5. Return to normal position. Repeat 5 times.

3 LATERAL TILT: Stand or sit erect. Slowly bend head to the side bringing the left ear to the left shoulder. Hold position to the count of 5. Return to normal position and repeat to opposite side. Repeat 5 times.

CARNRICK LABORATORIES
Division of Elan Pharmaceuticals
45 Horse Hill Road
Cedar Knolls, NJ 07927

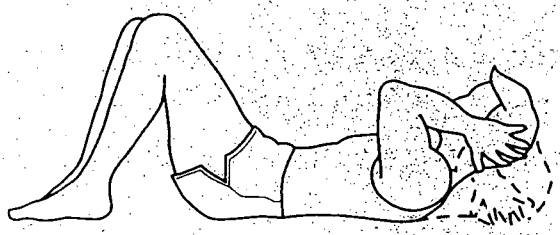
Strengthening Exercises

Quadriceps Bench Step



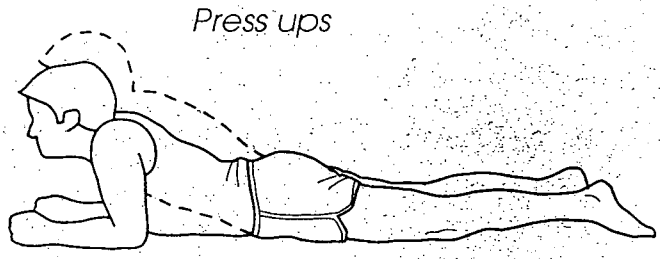
Bench Step
Use a 4-6" stool to step up and down. Repeat 10 times with each leg.

Abdomen



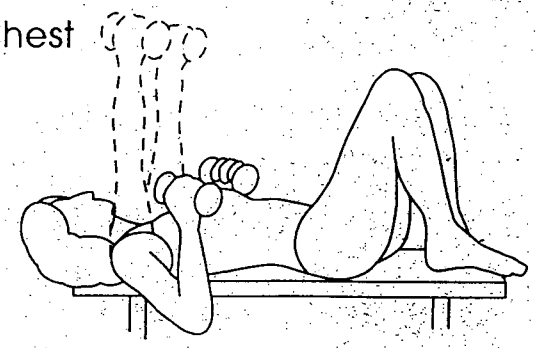
Crunches
With hands beside head, tilt pelvis to flatten back. Raise shoulders and head from floor, keeping 3 inches between chin and chest. Repeat 10 times.

Back



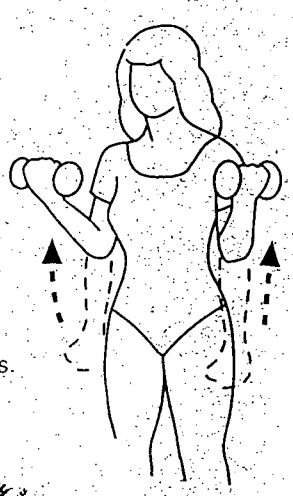
Press ups
Raise up on elbows as high as possible, keeping hips on floor. Repeat ___ times.

Chest



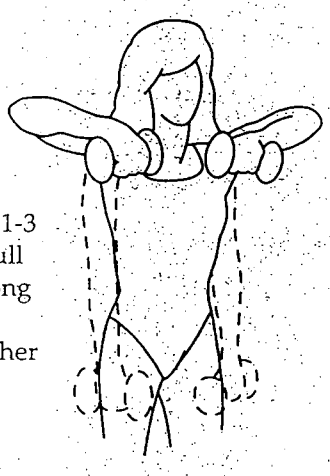
Bench Press
Use food cans or 1-3 lb. hand weights. Push straight up to full elbow extensions. Repeat 8-12 times.

Biceps



Arm Curls
Use cans of food or 1-3 lb. hand weights. With arm extended, pull weight to shoulder. Repeat 8-12 times.

Shoulders



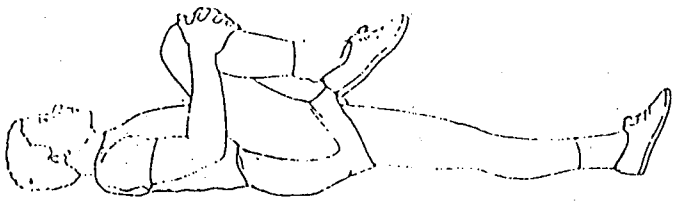
Upright Rowing
Use cans of food or 1-3 lb. hand weights. Pull weights upward along abdomen and chest until elbows are higher than chest.

XERCISE TREATMENT FOR BACK PAIN

)eveloped by Allan M. Levy, M.D.

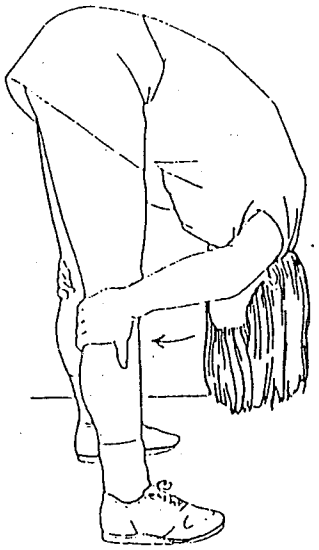
)·KNEE PULL

ie flat on your back and grasp one knee with interlaced ngers. Keeping your back flat, pull your knee toward your hest and hold for 20 seconds. Switch to the other knee nd repeat the stretch.



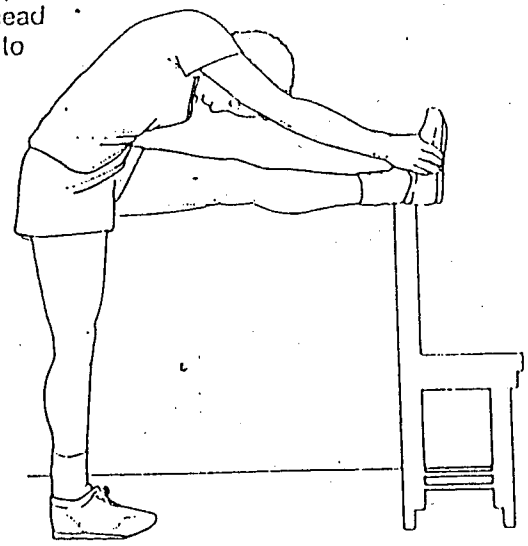
) FOREHEAD TO KNEE STRETCH

ower your forehead between your knees while standing. So as far as you can go, and ien grasp behind your nees and try to go a little arther. Hold for 10 to 15 econds. Start with three epetitions and then increase radually by one every other lay until you reach 12 epetitions.



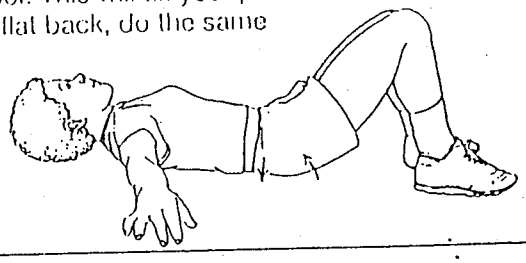
3) HURDLER STRETCH (STANDING)

While standing, put one foot on a chair in front of you. Now bend your forehead forward and try to touch it to your knee. Use the same number of repetitions as for the forehead to knee stretch. Repeat with the other leg.



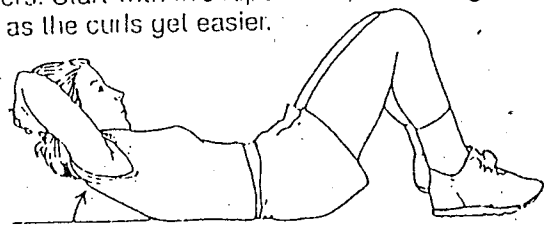
4) PELVIC TILT

While lying on your back with knees bent and feet flat on the floor, relax the back muscles and tighten your abdominal and buttock muscles to press your back flat against the floor. This will tilt your pelvis forward. Once you have a totally flat back, do the same number of repetitions as for the forehead to knee stretch.

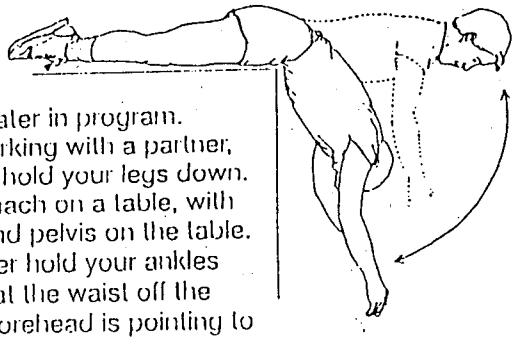


5) ABDOMINAL CURL

Lie on your back, knees bent and feet flat on the floor, with your hands clasped behind your head. Slowly curl your shoulder blades up off the floor, leaving your back on the floor. Hold for five seconds and slowly lower your head and shoulders. Start with five repetitions, increasing the number by five as the curls get easier.



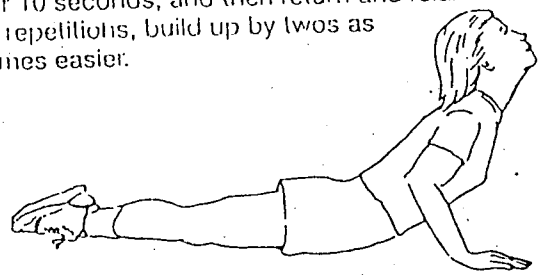
6) REVERSE SIT UP



This should not be started until later in program. This involves working with a partner, who will have to hold your legs down. Lie on your stomach on a table, with only your legs and pelvis on the table. Have your partner hold your ankles while you bend at the waist off the edge until your forehead is pointing to the floor. Then slowly lift your upper body until it is horizontal again. Do five repetitions and add two at a time as this becomes easier.

7) BACK EXTENSION

Lie on your stomach. Raise your head and shoulders. Hold for 10 seconds, and then return and relax. Do five repetitions, build up by twos as it becomes easier.



BREATHING EXERCISES

The feeling of not being able to get enough air into your lungs is frightening. Shortness of breath or difficulty breathing—called dyspnea—is a problem for people with chronic lung diseases. However, there are several things you can do to help you breathe more easily.

Avoiding Trouble

Breathing pollutants can aggravate dyspnea. Avoid heavy traffic and smog as much as possible. Don't use aerosol sprays. Stay away from products that produce fumes, such as paint, kerosene, and cleaning agents.

Cold weather can trigger dyspnea. If you must go outside when it's cold, cover your mouth with a scarf or mask.

Very dry air increases dyspnea and thickens the mucus in your lungs. A portable room humidifier is helpful, especially in the winter.

Physical exertion brings on dyspnea. Learn to conserve your energy by resting frequently, alternating light and heavy tasks, and minimizing movement. Instead of standing, sit. Instead of pushing or lifting objects, pull. Be creative in managing tasks—for example, a cart or child's wagon can be used to haul groceries, and wheels can be installed on furniture that is frequently moved.

Breathing Exercises

There are two simple exercises that can help you breathe more easily. You can do pursed lip breathing anywhere. With abdominal breathing, you will need to lie down. Practice them daily so when you are having problems with dyspnea, you will immediately know what to do.

Pursed Lip Breathing

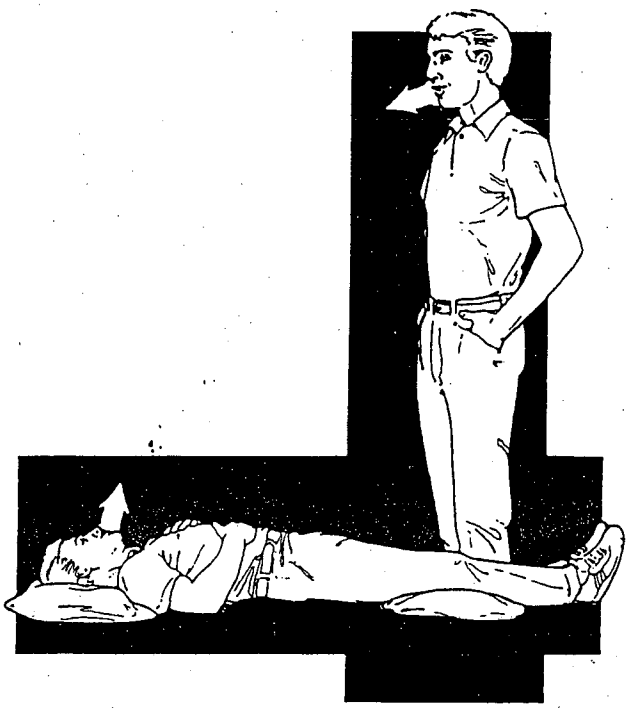
Pursed lip breathing will help get rid of the stale air trapped inside the lungs. It will slow down your breathing so it is more efficient. (Breathing fast only makes the dyspnea worse.)

1. Breathe in slowly through your nose. Hold your breath for three seconds (count to yourself by saying one 100, two 100, three 100). Be sure to breathe through your nose to avoid gulping air.
2. Purse your lips as if you were going to whistle or give someone a kiss.
3. Breathe out slowly through your pursed lips for six seconds (count one 100, two 100, three 100, four 100, five 100, six 100.) The sound you make breathing out will be like a soft whistle.

Abdominal Breathing

Abdominal breathing will also slow down your breathing to make it more effective. It also helps relax your entire body and is a wonderful technique to use before you go to sleep.

1. Lie on your back in a comfortable position with a pillow under your head. Place another pillow under your knees to help relax your abdomen.
2. Rest one hand on your abdomen just below your rib cage. Rest the other hand on your chest.
3. Slowly breathe in and out through your nose using your abdominal muscles. The hand resting on your abdomen will rise when you breathe in, and it will fall when you breathe out. The hand on your chest should be almost still.



whenever you stretch, you should ideally first spend several minutes engaged in some form of easy exercise—such as walking or performing light calisthenics—that will warm your muscles without stretching them. Or if you're preparing for exercise, you can warm up by gently going through the motions; for example, lob a few tennis balls over the net, shuffle around the track, lift some light weights.

The best technique for most people is **static stretching**, which involves gradually extending a muscle just to the point of discomfort—never beyond—and then holding it there for 15 to 30 seconds. **Ballistic stretching**—working a muscle with rapid, repeated, bouncing motions—may be appropriate for certain individuals, such as martial artists and ballet dancers, who need to develop dynamic flexibility. But it's too dangerous for most folks, since it can force a muscle to extend just as it's contracting, which can lead to pulls or tears. Two other methods—**contract-relax stretching** and **active isolated stretching**—have gained some converts of late, but offer no clear advantage over static stretching.

Ten minutes of stretching every other day or so will help you maintain overall flexibility. (See our sample stretching program below.) You should also allow a full 10 minutes for stretching if you re about to engage in an activity that requires abrupt changes in speed and direction—such as baseball, basketball, soccer, or any racquet sport. Such stop-and-go activities are most likely to cause muscle pulls. On the other hand, a couple of minutes of stretching will suffice if you're preparing for a steadier sort of activity, such as walking, jogging, hiking, bicycling, or swimming.

It's wise to take a minute to stretch now and then during physical activity as well—particularly if that activity involves periods of inactivity. Stretching during those down times helps prevent injury and muscle soreness by keeping the muscles from tightening up.

Another round of stretching after you're finished exercising will likewise help prevent muscle soreness. Follow that same 2-to-10-minute rule, depending on the nature of your workout. That's also a good time to promote overall flexibility, since your muscles will be at their warmest, allowing for maximum extension in a stretch.

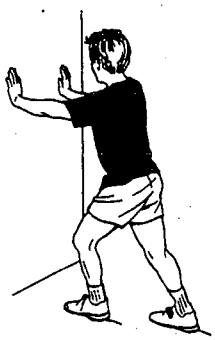
All-purpose stretching routine

If you're stretching to prepare for a particular sport or other form of exercise, you can concentrate on the stretches that work the muscles you'll be using most. Swimmers, for example, can concentrate on the groin and shoulder/back stretches (numbers 1 and 6). Bicyclists and joggers would benefit most from stretching the groin, hamstrings, calves, and quadriceps (1, 2, 4, and 7). If you're preparing for tennis or some other racquet sport, you should work on all seven of the stretches.

Move into each stretch slowly, stopping just at the point of discomfort. Hold there for 15 to 30 seconds and relax. Repeat each stretch two or three times.

Groin stretch. Sit upright on the floor. With your knees flexed and the soles of your feet together, pull your feet toward your groin. Place your elbows on the insides of your thighs and exhale as you slowly push your knees toward the floor.

Hamstring stretch. Sit on the floor with one leg extended. Reach forward along that leg until you feel a gentle stretch in your hamstring, the muscles at the back of your thigh. Flex from the hips, keeping your back straight.



Calf stretch. Place your palms against a wall at shoulder height. Step forward with one foot, bending that knee, and extend the other leg behind you, toes pointed straight ahead. Keeping both heels on the floor, lean forward until you feel the stretch in the calf of your extended leg.

Single leg pull. Lie on your back with one knee bent and the other leg flat on the floor. Flex both feet so your toes point upward. Grip your bent leg just behind the knee and gently pull toward your chest until you feel the stretch in your buttocks and lower back.

Triceps stretch. Sit or stand upright with one arm flexed and raised so that the upper arm lies against the side of your head; let that hand drop down to rest on your opposite shoulder blade. Now, use the other hand to grasp that raised elbow, and exhale as you pull the elbow behind your head.

Shoulder/back stretch. Lock your fingers above your head, with palms facing upward. Push your arms slightly back and up until you feel the stretch in your shoulders, upper back, and arms.

Standing quadriceps stretch. Stand straight with knees slightly bent. Using one hand to hold the back of a chair or some other object to keep your balance, lift one leg behind you and grasp that foot with your free hand. Gently pull the foot upward toward your buttocks until you feel the stretch along your quadriceps, the muscle on the front of the thigh.

TETANUS, DIPHTHERIA (Td) *or* TETANUS, DIPHTHERIA, PERTUSSIS (Tdap) VACCINE

WHAT YOU NEED TO KNOW

Many Vaccine Information Statements are available in Spanish and other languages. See www.immunize.org/vis.

1 Why get vaccinated?

Children 6 years of age and younger are routinely vaccinated against tetanus, diphtheria and pertussis. But older children, adolescents, and adults need protection from these diseases too. Td (Tetanus, Diphtheria) and Tdap (Tetanus, Diphtheria, Pertussis) vaccines provide that protection.

TETANUS (Lockjaw) causes painful muscle spasms, usually all over the body.

- It can lead to tightening of the jaw muscles so the victim cannot open his mouth or swallow. Tetanus kills about 1 out of 5 people who are infected.

DIPHTHERIA causes a thick covering in the back of the throat.

- It can lead to breathing problems, paralysis, heart failure, and even death.

PERTUSSIS (Whooping Cough) causes severe coughing spells, vomiting, and disturbed sleep.

- It can lead to weight loss, incontinence, rib fractures and passing out from violent coughing. Up to 2 in 100 adolescents and 5 in 100 adults with pertussis are hospitalized or have complications, including pneumonia.

These three diseases are all caused by bacteria. Diphtheria and pertussis are spread from person to person. Tetanus enters the body through cuts, scratches, or wounds.

The United States averaged more than 1,300 cases of tetanus and 175,000 cases of diphtheria each year before vaccines. Since vaccines have been available, tetanus cases have fallen by over 96% and diphtheria cases by over 99.9%.

Before 2005, only children younger than than 7 years of age could get pertussis vaccine. In 2004 there were more than 8,000 cases of pertussis in the U.S. among adolescents and more than 7,000 cases among adults.

2 Td and Tdap vaccines

- Td vaccine has been used for many years. It protects against tetanus and diphtheria.
- Tdap was licensed in 2005. It is the first vaccine for adolescents and adults that protects against all three diseases.

> Note: At this time, Tdap is licensed for only one lifetime dose per person. Td is given every 10 years, and more often if needed.

These vaccines can be used in three ways: 1) as catch-up for people who did not get all their doses of DTaP or DTP when they were children, 2) as a booster dose every 10 years, and 3) for protection against tetanus infection after a wound.

3 Which vaccine, and when?

Routine: Adolescents 11 through 18

- A dose of Tdap is recommended for adolescents who got DTaP or DTP as children and have not yet gotten a booster dose of Td. The preferred age is 11-12.
- Adolescents who have already gotten a booster dose of Td are encouraged to get a dose of Tdap as well, for protection against pertussis. Waiting at least 5 years between Td and Tdap is encouraged, but not required.
- Adolescents who did not get all their scheduled doses of DTaP or DTP as children should complete the series using a combination of Td and Tdap.

Routine: Adults 19 and Older

- All adults should get a booster dose of Td every 10 years. Adults under 65 who have never gotten Tdap should substitute it for the next booster dose.
- Adults under 65 who expect to have close contact with an infant younger than 12 months of age (including women who may become pregnant) should get a dose of Tdap. Waiting at least 2 years since the last dose of Td is suggested, but not required.
- Healthcare workers under 65 who have direct patient contact in hospitals or clinics should get a dose of Tdap. A 2-year interval since the last Td is suggested, but not required.

New mothers who have never gotten Tdap should get a dose as soon as possible after delivery. If vaccination is needed *during* pregnancy, Td is usually preferred over Tdap.

Protection After a Wound

A person who gets a severe cut or burn might need a dose of Td or Tdap to prevent tetanus infection. Tdap may be used for people who have never had a dose. But Td should be used if Tdap is not available, or for:

- anybody who has already had a dose of Tdap,
- children 7 through 9 years of age, or
- adults 65 and older.

Tdap and Td may be given at the same time as other vaccines.

4 Some people should not be vaccinated or should wait

- Anyone who has had a life-threatening allergic reaction after a dose of DTP, DTaP, DT, or Td should not get Td or Tdap.
- Anyone who has a severe allergy to any component of a vaccine should not get that vaccine. Tell your provider if the person getting the vaccine has any severe allergies.

- Anyone who had a coma, or long or multiple seizures within 7 days after a dose of DTP or DTaP should not get Tdap, unless a cause other than the vaccine was found (these people *can* get Td).
- Talk to your provider if the person getting either vaccine:
  - has epilepsy or another nervous system problem,
  - had severe swelling or severe pain after a previous dose of DTP, DTaP, DT, Td, or Tdap vaccine, or
  - has had Guillain Barré Syndrome (GBS).

Anyone who has a moderate or severe illness on the day the shot is scheduled should usually wait until they recover before getting Tdap or Td vaccine. A person with a mild\ illness or low fever can usually be vaccinated.

5 What are the risks from Tdap and Td vaccines?

With a vaccine (as with any medicine) there is always a small risk of a life-threatening allergic reaction or other serious problem.

Getting tetanus, diphtheria or pertussis would be much more likely to lead to severe problems than getting either vaccine.

Problems reported after Td and Tdap vaccines are listed below.

Mild Problems
(Noticeable, but did not interfere with activities)

Tdap
- Pain (about 3 in 4 adolescents and 2 in 3 adults)
- Redness or swelling (about 1 in 5)
- Mild fever of at least 100.4°F (up to about 1 in 25 adolescents and 1 in 100 adults)
- Headache (about 4 in 10 adolescents and 3 in 10 adults)
- Tiredness (about 1 in 3 adolescents and 1 in 4 adults)
- Nausea, vomiting, diarrhea, stomach ache (up to 1 in 4 adolescents and 1 in 10 adults)
- Chills, body aches, sore joints, rash, swollen glands (uncommon)

Td
- Pain (up to about 8 in 10)
- Redness or swelling (up to about 1 in 3)
- Mild fever (up to about 1 in 15)
- Headache or tiredness (uncommon)

Moderate Problems
(Interfered with activities, but did not require medical attention)

Tdap
- Pain at the injection site (about 1 in 20 adolescents and 1 in 100 adults)
- Redness or swelling (up to about 1 in 16 adolescents and 1 in 25 adults)
- Fever over 102°F (about 1 in 100 adolescents and 1 in 250 adults)
- Headache (1 in 300)
- Nausea, vomiting, diarrhea, stomach ache (up to 3 in 100 adolescents and 1 in 100 adults)

Td
- Fever over 102°F (rare)

Tdap or Td
- Extensive swelling of the arm where the shot was given (up to about 3 in 100).

Severe Problems
(Unable to perform usual activities; required medical attention)

Tdap
- Two adults had nervous system problems after getting the vaccine during clinical trials. These may or may not have been caused by the vaccine. These problems went away on their own and did not cause any permanent harm.

Tdap or Td
- Swelling, severe pain, and redness in the arm where the shot was given (rare).

A severe allergic reaction could occur after any vaccine. They are estimated to occur less than once in a million doses.

6 What if there is a severe reaction?

What should I look for?
Any unusual condition, such as a high fever or behavior changes. Signs of a severe allergic reaction can include difficulty breathing, hoarseness or wheezing, hives, paleness, weakness, a fast heart beat or dizziness.

What should I do?
- Call a doctor, or get the person to a doctor right away.
- Tell the doctor what happened, the date and time it happened, and when the vaccination was given.
- Ask your provider to report the reaction by filing a Vaccine Adverse Event Reporting System (VAERS) form. Or you can file this report through the VAERS website at **www.vaers.hhs.gov**, or by calling **1-800-822-7967**.

VAERS does not provide medical advice.

7 The National Vaccine Injury Compensation Program

A federal program exists to help pay for the care of anyone who has a serious reaction to a vaccine.

For details about the National Vaccine Injury Compensation Program, call **1-800-338-2382** or visit their website at **www.hrsa.gov/vaccinecompensation**.

8 How can I learn more?

- Ask your provider. They can give you the vaccine package insert or suggest other sources of information.
- Call your local or state health department.
- Contact the Centers for Disease Control and Prevention (CDC):
  - Call **1-800-232-4636 (1-800-CDC-INFO)** or
  - Visit CDC's website at **www.cdc.gov/vaccines**.

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR DISEASE CONTROL AND PREVENTION

PNEUMOCOCCAL CONJUGATE VACCINE

WHAT YOU NEED TO KNOW

Many Vaccine Information Statements are available in Spanish and other languages. See http://www.immunize.org/vis.

1 Pneumococcal disease

Infection with *Streptococcus pneumoniae* bacteria can make children very sick.

It causes blood infections, pneumonia, and meningitis, mostly in young children. (Meningitis is an infection of the covering of the brain.) Although pneumococcal meningitis is relatively rare (less than 1 case per 100,000 people each year), it is fatal in about 1 of 10 cases in children.

Pneumococcal meningitis can also lead to other health problems, including deafness and brain damage.

Before routine use of pneumococcal conjugate vaccine, pneumococcal infections caused:

- over 700 cases of meningitis,
- 13,000 blood infections,
- about 5 million ear infections, and
- about 200 deaths

annually in the United States in children under five.

Children younger than 2 years of age are at higher risk for serious disease than older children.

Pneumococcal bacteria are spread from person to person through close contact.

Pneumococcal infections may be hard to treat because some strains of the bacteria have become resistant to the drugs that are used to treat them. This makes **prevention** of pneumococcal infections through vaccination even more important.

2 Pneumococcal conjugate vaccine (PCV13)

There are more than 90 types of pneumococcal bacteria. The new pneumococcal conjugate vaccine (PCV13) protects against 13 of them. These bacteria types are responsible for most severe pneumococcal infections among children. PCV13 replaces a previous conjugate vaccine (PCV7), which protected against 7 pneumococcal types and has been in use since 2000. During that time severe pneumococcal disease dropped by nearly 80% among children under 5.

PCV13 may also prevent some cases of pneumonia and some ear infections. But pneumonia and ear infections have many causes, and PCV13 only works against the types of pneumococcal bacteria targeted by the vaccine.

PCV13 is given to infants and toddlers, to protect them when they are at greatest risk for serious diseases caused by pneumococcal bacteria.

In addition to receiving PCV13, older children with certain chronic illnesses may get a different vaccine called PPSV23. There is a separate Vaccine Information Statement for that vaccine.

3 Who should get PCV13, and when?

Infants and Children Under 2 Years of Age

PCV13 is recommended as a series of **4 doses**, one dose at each of these ages: 2 months, 4 months, 6 months, and 12 through 15 months

Children who miss their shots at these ages should still get the vaccine. The number of doses and the intervals between doses will depend on the child's age. Ask your health care provider for details.

Children who have begun their immunization series with PCV7 should complete the series with PCV13.

Older Children and Adolescents

- Healthy children between their 2nd and 5th birthdays who have not completed the PCV7 or PCV13 series before age 2 years should get 1 dose.

- Children between the 2nd and 6th birthdays with medical conditions such as:
  - sickle cell disease,
  - a damaged spleen or no spleen,
  - cochlear implants,
  - diabetes,
  - HIV/AIDS or other diseases that affect the immune system (such as cancer, or liver disease), or
  - chronic heart or lung disease,

  or who take medications that affect the immune system, such as immunosuppressive drugs or steroids, should get **1 dose of PCV 13** (if they received 3

doses of PCV7 or PCV13 before age 2 years), or **2 doses of PCV13** (if they have received 2 or fewer doses of PCV7 or PCV13).

A dose of PCV13 may be administered to children and adolescents 6 through 18 years of age who have certain medical conditions, even if they have previously received PCV7 or PPSV23.

Children who have completed the 4-dose series with PCV7: Healthy children who have not yet turned 5, and children with medical conditions who have not yet turned 6, should get one additional dose of PCV13.

Ask your health care provider if you have questions about any of these recommendations.

PCV13 may be given at the same time as other vaccines.

4 Some children should not get PCV13 or should wait

Children should not get PCV13 if they had a serious (life-threatening) allergic reaction to a previous dose of this vaccine, to PCV7, or to any vaccine containing diphtheria toxoid (for example, DTaP).

Children who are known to have a severe allergy to any component of PCV7 or PCV13 should not get PCV13. Tell your health care provider if your child has any severe allergies.

Children with minor illnesses, such as a cold, may be vaccinated. But children who are moderately or severely ill should usually wait until they recover before getting the vaccine.

5 What are the risks from PCV13?

Any medicine, including a vaccine, could possibly cause a serious problem, such as a severe allergic reaction. However, the risk of any vaccine causing serious harm, or death, is extremely small.

In studies, most reactions after PCV13 were mild. They were similar to reactions reported after PCV7, which has been in use since 2000. Reported reactions varied by dose and age, but on average:

- About half of children were drowsy after the shot, had a temporary loss of appetite, or had redness or tenderness where the shot was given.
- About 1 out of 3 had swelling where the shot was given.
- About 1 out of 3 had a mild fever, and about 1 in 20 had a higher fever (over 102.2°F).
- Up to about 8 out of 10 became fussy or irritable.

Life-threatening allergic reactions from vaccines are very rare. If they do occur, it would be within a few minutes to a few hours after the vaccination.

6 What if there is a severe reaction?

What should I look for?

Any unusual condition, such as a high fever or behavior changes. Signs of a severe allergic reaction can include difficulty breathing, hoarseness or wheezing, hives, paleness, weakness, a fast heart beat or dizziness.

What should I do?

- Call a doctor, or get the person to a doctor right away.
- Tell the doctor what happened, the date and time it happened, and when the vaccination was given.
- Ask your provider to report the reaction by filing a Vaccine Adverse Event Reporting System (VAERS) form.
  Or you can file this report through the VAERS website at: **http://www.vaers.hhs.gov**, or by calling **1-800-822-7967**.

VAERS does not provide medical advice.

7 The National Vaccine Injury Compensation Program

The National Vaccine Injury Compensation Program (VICP) was created in 1986.

Persons who believe they may have been injured by a vaccine may file a claim with VICP by calling **1-800-338-2382** or visiting their website at **http://www.hrsa.gov/vaccinecompensation**.

8 How can I learn more?

- Ask your provider. They can give you the vaccine package insert or suggest other sources of information.
- Call your local or state health department.
- Contact the Centers for Disease Control and Prevention (CDC):
  - Call **1-800-232-4636 (1-800-CDC-INFO)** or
  - Visit CDC's website at **http://www.cdc.gov/vaccines**.

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR DISEASE CONTROL AND PREVENTION